

Treatment and Exercises

IFS, which explicitly welcomes and deals with all symptoms (or, in the language of IFS, protective parts) from the outset, is typically an accelerated form of treatment. The open, accepting stance of IFS promotes inner safety, which invites early access to vulnerability. As protective parts begin to feel known and seen by the client's Self, which can occur within two or three sessions, they tend to relax and a client with, for instance, mild to moderate depression would experience significant relief.

In the IFS Complex Trauma Study, only one subject out of 13 still qualified for a diagnosis of PTSD after finishing 16 weeks of IFS therapy. That said, it can take weeks to months for protectors to grant the client's Self permission to be with wounded parts, depending on the severity of their injuries and the skill level of the therapist. Once granted, however, the process of the client's Self witnessing the experiences of the wounded part and helping it to unburden is usually completed within one to three sessions.

CLIENT SELECTION

IFS has been used effectively with a wide variety of mental health conditions, including but not limited to: trauma, dissociation, depression, panic, anxiety, eating disorders, addictions, OCD, ADD, bipolar disorder, personality disorders and schizophrenia. It is effective with adults, children and individuals with special needs such as autism. It has been adapted for couples therapy, group therapy, child therapy and family therapy. The main caveat involves safety: If the client's living environment is acutely dangerous or unsafe, encouraging protective parts to relax so she can connect to vulnerability (injured, exiled parts) could make her less safe. In this case we focus on safety first.

SWITCHING TO IFS TREATMENT WITH AN ONGOING CLIENT

Most therapists come to IFS with an ongoing clientele and expertise in at least one other approach to therapy. If this is the case for you, we suggest giving some thought to which of your clients might be most open to trying something new and then introducing the model with an invitation to try it out. Such an introduction might go something like this.

THERAPIST: Remember I went to that training a couple of weeks ago to learn about this new kind of therapy? I was impressed. I thought of you and wondered if you'd like to try it.

LINA: Why did you think of me?

THERAPIST: It would be easier for me to explain that if I tell you more about the approach. Okay?

LINA: Sure.

THERAPIST: For me the cool thing about this therapy, which is called internal family systems or IFS for short, is that it focuses on how we all have many different things going on inside, which we do talk about here a lot. Like you have that part who really wanted to have a child before it was too late and then you also have an impatient part who can get so disappointed with Mickey, like when he was a colicky baby and now when he comes home from preschool acting so needy and clings to your leg. Right? And then you have a part who feels guilty about you being impatient. That part calls you a bad mother and says you should never have had a child. Right?

LINA: Yeah but we know all that. So what's different?

THERAPIST: Here's what's different, which I never knew before. Rather than talking about these parts we can actually talk to them. Want to try?

LINA: I don't know. Am I going to be your guinea pig?

THERAPIST: You're definitely going to be my guinea pig. But I wouldn't suggest it if I didn't think there was something in it for you.

LINA: If I talk to these parts and they talk back maybe they'll never shut up and that'll make my life even worse.

THERAPIST: I can understand that concern. So, although I'm just learning, this IFS training is experiential and I did it myself for a few days. I listened to my parts and I talked to them and we communicated back and forth. Like imagine if you had gone to family therapy with your parents and sisters when you were a kid. If they had all given you the opportunity to speak up, and had listened and given you the recognition you needed so much, would that have made you more or less able to listen to them?

LINA: More.

THERAPIST: Yeah. And it's the same for parts. It turns out, just like people, they want to be heard and they want their concerns to be respected. It makes them more able to take turns, not less.

LINA: So are you going to show me now?

THERAPIST: Yep. I'll show you. And, since you're the expert on you, if you like it we can learn together.



As the example illustrates, clients (and their parts) have expectations about the nature and purpose of therapy and will generally have legitimate concerns about taking a new tack. Schwartz calls IFS therapists “hope merchants” because, from the beginning to the end of a therapy, we are selling parts on the idea of trying something new. This role is particularly important if we want to persuade ongoing clients to try IFS.

STARTING IFS TREATMENT WITH A NEW CLIENT

Occasionally clients seek out a specific kind of treatment such as IFS, but more often they come to therapy because they want to feel better. At the outset of therapy some clients just want to see if the fit with this particular therapist feels good; others enter therapy with questions in hand about the therapist's theoretical orientation and style. After hearing about IFS, these latter

clients may accept the idea of psychic multiplicity. On the other hand, they may be puzzled or concerned. The idea that parts are normal but some have been wounded while others are protective – and that all have good intentions – tends to be novel. When we encourage clients to “go inside,” listen to their parts and connect with their Self, some will be wary while others will simply be willing. In our experience, the more we practice the more we get used to a range of responses and the more comfortable we feel responding according to the individual client’s needs.

INTRODUCING KEY IFS CONCEPTS

WHEN STARTING WITH A NEW CLIENT

Here is an example of how one might talk about parts and the Self when starting with a new client who wants to be oriented to your approach to therapy.

THERAPIST: Nice to meet you, Logan. Tell me why you're here and how I might help.

LOGAN: Well school has been a big drag lately. My classes are harder this semester and I feel stressed and overwhelmed. Although my girlfriend keeps trying to help me write papers, we fight all the time. It's all just super pissing me off.

THERAPIST: Sounds challenging. I'm glad you're here.

LOGAN: Yeah my parents have been harping on me to go to therapy and figure out what's wrong with me.

THERAPIST: Well, let me say upfront that I wouldn't take the view that something is wrong with you. I hear you're in a difficult place and you're trying, as best you can, to help yourself.

- *The therapist asserts a non-pathologized view of Logan's distress.*

LOGAN: Thank you! That's what I think. So what do you do?

THERAPIST: My approach to therapy is called internal family systems, or IFS.

LOGAN: My girlfriend is a psych major. What is IFS?

THERAPIST: Well, IFS is quite different. In my experience, the best introduction is direct experience - I can show you.

LOGAN: You can't tell me anything?

THERAPIST: Sure I can. This approach to therapy says that we all have lots of different parts. For example, I'm different here at work than I am at home when I play with my kids or than I am when I'm running a road race with friends. These parts are

like different aspects of my personality. And I have lots more, too. Do you know what I mean?

LOGAN: I guess.

THERAPIST: Maybe it would help if I tell you what I've heard about your parts? Because in this way we're all the same. We all have parts. For example, you spoke about a part who feels overwhelmed and stressed out about school, and a part who is fighting with your girlfriend and generally seems angry all the time. As well, you mentioned having a part who disagrees with your parents that something is wrong with you. And clearly you have a part who was willing to come to therapy even though you disagree with your parents. Is that right?

LOGAN: Yes.

THERAPIST: So we all have parts and our parts take on certain roles and responsibilities to help us get through difficult times. And here's what's different about IFS. We can talk with your parts directly, if you want.

- *The therapist leaves the decision to proceed in Logan's hands, de facto asking for permission to proceed.*

LOGAN: That sounds good. But I'm afraid if I think about all this too much I'll be more bummed out when I leave.

- *Although one part is now intrigued, another - a concerned part - speaks up.*

THERAPIST: I understand. I don't want you to leave here feeling more bummed out.

Can I tell you one more thing that might help?

LOGAN: Okay.

THERAPIST: In addition to lots of parts, we all also have internal strength to get through tough times. So deep down I'd say you do know what's best and you have the inner resources to navigate this situation. These resources include a kind of inner wisdom that is your core Self: the Logan who is not a part. I can help your parts meet that Logan if they want.

- *The therapist introduces the idea of the Self.*

LOGAN: They don't think it's true.

THERAPIST: I'm glad they spoke up because I wouldn't want them to take my word for it. And I want them to know that meeting the Logan who is not a part would not involve them changing or giving anything up, it would just be to learn about what that Logan has to offer.

LOGAN: Okay I'll try.

THERAPIST: So let's start with one part. Who needs your attention first?

- *Always have a target part.*

LOGAN: How about the one who's stressed out by school?

THERAPIST: Good. Turn your attention inside – you can close your eyes if you want, or not. Then notice where you find that feeling of stress in, on or around your body.

LOGAN: In my stomach. I feel kind of sick all the time.

THERAPIST: How do you feel toward that sick sensation?

LOGAN: Honestly I wish it would stop. I know I may not get A's but I'd like to get through this term without losing my mind.

- *This is another part.*

THERAPIST: So the one who wants it to stop sounds like another part who has an important perspective. But would it be willing to let you hear from the stressed part first if we ask it not to overwhelm you?

- *The therapist asks this other part for permission to proceed with the target part.*

LOGAN: Okay.

THERAPIST: Let it know that you want to help but if it takes you over, you won't be there to help. Is it willing to not overwhelm you?

LOGAN: Yes.

THERAPIST: Good. How do you feel toward the stressed part now?

- *As we illustrate throughout this manual, "how do you feel toward" is a key question for assessing how open the client is to hearing from a target part, which is the same as determining the client's level of Self-energy.*

LOGAN: Well kinda curious. I know school sucks right now, but I've been through this before. Why should it make me feel so sick and angry now?

- *Logan's answer shows that he is now open-hearted enough to listen.*

THERAPIST: How does the part respond?

LOGAN: It's shouting, *You can't fail!*

THERAPIST: Is it okay to hear more?

LOGAN: Yes.

THERAPIST: Would it be willing to speak rather than shouting?

- *The therapist begins to normalize communication between the client and his parts: no need to shout. The more Logan's Self is available, the calmer this part will be.*

LOGAN: If I listen.

THERAPIST: Are you ready to listen?

- *The therapist does not take this for granted, he is checking to see if other parts are willing to let Logan listen to the distressed part.*

LOGAN: Yes.

THERAPIST: Okay. Let it know you're listening so it doesn't need to shout. Ask it to tell you more about why you can't fail.



Logan wants a conceptual framework so the therapist introduces the basic concepts of IFS. The client, in turn, is able to feel curious about his current experience, which has been so negative and preoccupying. Not all initial sessions will cover this much territory, which is fine. Pacing in IFS is indexed to the needs, willingness and permission of protective parts: we go as fast as they let us go.

WINNING BUY-IN WITH A NEW CLIENT WHO ISN'T INTERESTED IN THE THEORY OF IFS

Here is an example of starting with a new client who does not ask to be oriented to the therapist's approach to therapy. He is in significant emotional pain but, because he believed he should be able to "figure this out" by himself, had been avoiding therapy.

THERAPIST: Welcome, Rory. On the phone you explained that you and your girlfriend have not been getting along, that the two of you have a 3-year-old daughter, and that they have gone to stay with her mother out in Washington State for an indefinite period of time. You also said you've been having trouble sleeping, which interferes with your ability to focus at work. Which of these concerns needs your attention first?

RORY: I really thought I could - I should be able to - handle all this myself. I didn't want to see a therapist.

THERAPIST: Should we pay attention to that?

RORY: I guess so.

THERAPIST: So let me see if I have this right, one part of you thought you should be able to handle these problems on your own but another part urged you to get help?

RORY: Yes.

THERAPIST: Which part needs your attention first?

RORY: The part that thinks I should do it on my own is really strong right now.

THERAPIST: Where do you notice that in, on or around your body?

RORY: In my head.

THERAPIST: How do you feel toward it?

RORY: I agree.

THERAPIST: So it sounds like right at this moment you're seeing things through the eyes of the part who thinks you should be able to handle all this alone. But we also know you have that other part - the one who thinks it's okay for you to get help. Where is it right now?

RORY: Out here.

- *Rory waves his hand behind his head.*

THERAPIST: I see. That one's in orbit. Would the one who's in your head be willing to make room for you inside?

RORY: Yes.

THERAPIST: So there's a Rory in there who has some wisdom and can be really helpful. With your permission, I'd like to introduce you to him. Is that okay? Good. Now ask the part who says you should do this alone to relax for a moment and don't think, just listen ... What do you hear?

RORY: I know I'm always supposed to be strong and independent, but it's okay for me to be here. I need to be a good father. This is important and I have to talk about it.

THERAPIST: Is that the part who brought you here?

RORY: I think so.

THERAPIST: Ask.

RORY: Yes.

THERAPIST: Is it still out there?

RORY: It's closer now.

THERAPIST: Would it be willing to let you talk first to the one who objects to you being here?

RORY: Okay.

THERAPIST: So how do you feel now toward the one who thinks you should do it alone?

RORY: Crowded.

THERAPIST: Would it make room for you?

RORY: How would I know?

THERAPIST: Just ask.

RORY: I don't hear anything but I do feel a bit more room inside.

THERAPIST: Great. Thank it. How do you feel toward it now?

RORY: I know where this came from. My dad thinks people who need help are weak.

THERAPIST: Let this part know what you know about that and ask if you've got it right.

RORY: Yes.

THERAPIST: What does it need from you?

RORY: It wants me to take care of this mess with Susan and be a good dad.

THERAPIST: Does that make sense to you?

RORY: That's what I'm here for.



Rory came to therapy because he was losing his relationship and his child, but not all of his parts were on board with this decision. The therapist helped him notice their inner disagreement and get into relationship with them without going into the theory of IFS. Some clients ask to know more while others just get engaged and slip into the practice of relating to parts without thinking about it.

THE ROLE OF LANGUAGE

If a client objects to using the word “part” we can easily denote parts by using whatever words the client uses: I’m angry, I freak out, I’m afraid, I feel totally bummed, etc. The key to IFS therapy is our confidence in the model, hence we emphasize the therapist getting firsthand, personal experience in IFS trainings and IFS therapy. As a general piece of advice, beginners will want to start with clients who are reasonably stable and who might be interested in the model.

OUTING PROTECTORS

And one last caveat on language: sometimes a part shows up who is less than thrilled to be “outed” in therapy. We are careful to respect these parts by asking if they are willing to talk. Here is an example:

CLIENT’S PART: I’m not a part. Marc doesn’t have parts.

THERAPIST: Okay. If he did have parts would that be a problem?

CLIENT’S PART: It would be if he had them.

THERAPIST: Is it okay to ask what the problem would be?

CLIENT’S PART: First of all, it’s sick. And, second, no one told me.

THERAPIST: I see! That’s true. I apologize. I didn’t mean to startle you. You’re right that no one asked you if it would be okay to talk about parts. I should have asked, but before I do, I want you to know that I don’t consider having parts an illness. From my perspective we all have lots of parts and that’s a good thing. So now, if it’s okay with you, I’ll ask. Can we talk about this?

- *Here the therapist moves into speaking directly with the part who is in the driver’s seat inside of Marc.*

CLIENT’S PART: You can say more.

THERAPIST: Well in my experience I have parts and so does everyone else. So if Marc had parts he would be totally within the norm. But whether or not we talk about parts is up to you and him.

- *The therapist speaks directly to this protective part (referring to Marc in the third person) to reassure it that she is not trying to pathologize Marc.*



As we see with this interaction, as well as guiding the client to speak to parts, we can always speak to parts directly as needed, a technique called Direct Access.

A TRAUMA-INFORMED THERAPY

Unlike most trauma-informed therapies, IFS is not divided into phases that focus on affect regulation and interpersonal skills that come before reviewing and integrating trauma memories. Instead “IFS starts from the premise that psychic parts constitute a motivated and purposeful inner community that does not need to be managed. We offer this community our interest and curiosity; in return the parts explain why they believe their behavior, which can seem consistently irrational and destructive to the observer, benefits the client.” (Anderson & Sweezy, 2016)

Nevertheless, we do have methodology and goals: to help protectors, help the exile and harmonize all in the internal system with Self-leadership. If we think of an exile and the protectors who surround it as constituting a discrete mental beehive (of which there are many) we see that protectors, who have tremendous willingness to self-sacrifice for safety and the greater good, are very much the worker bees of the psyche while the exile is their captive queen. In IFS we help these psychic beehives one at a time and our first steps (described below as “the 6Fs”) involve befriending the worker bees.

PROTECTORS SET THE PACE

IFS treatment begins by going through the 6F steps (p. 38) with protective parts, after which we move on to the “healing steps” of witnessing and unburdening exiles. To earn the trust of the inner system we often need to roll repeatedly through the steps described below, sometimes for weeks or months before protective parts give us permission to communicate with an exile directly. The more traumatized the system, the longer this may take.

GET TO KNOW YOUR SYSTEM FIRST

Before settling in with a target part for the first time, we recommend saying hello to your whole internal system. Here are two exercises and one meditation to introduce you to your parts.

- The first involves welcoming all parts.
- The second aims to help access Self-energy with protectors who have become unpopular in the system.
- The third is a meditation practice for going inside, noticing and being available to your parts. Each person experiences their parts differently, usually through one of the senses.

Parts can show us a thought, feeling or sensation. Some people hear their parts, some see their parts, while others feel their parts emotionally or physically.

ALL PARTS ARE WELCOME

The simplicity of the IFS credo “all parts are welcome” can belie the courage and Self-energy required to welcome all parts. As Schwartz wrote in 2013:

The Self-led therapist is implicitly issuing an invitation to the client: “All parts are welcome!” And from the darkest corners of their psyches come the hidden aspects of clients in all their crazy glory. And that’s a good thing. The inescapable reality of therapy is that if we do our job well some clients will do all kinds of provocative things... They’ll resist, get angry and critical, become hugely dependent, talk incessantly, behave dangerously between sessions, show intense vulnerability, idealize us, attack themselves, and display astounding narcissism and self-centeredness (Schwartz, 2013, p.11).

The ability to regain compassionate presence in the face of a client’s extreme protectors or exiles is the *sine qua non* of a good IFS therapist. Protectors do not need us to point that out that their attempts at warding off pain are always costly failures. They know it. Instead of dwelling on the effects of their job, we ask about their goals. What do they want for the client? And we appreciate their positive intentions as well as all their hard work.

Exercise

ALL PARTS ARE WELCOME

DIRECTIONS: In IFS our credo is: All parts are welcome! Here is an exercise to help you welcome all of your parts.

Turn your attention inside and begin with this offer:

“I want to help anyone who needs help. To do that, I need to know all of you.”

Then provide this information:

“If you overwhelm me, I can’t be there to help you.”

And make this request:

“Please be here with me rather than taking me over and, when you’re ready, let me know who you are. I will write this down.”

Write down the parts (thoughts, feelings or sensations) that you hear, see or sense inside (use extra paper if needed):

Exercise

GETTING TO KNOW UNPOPULAR PARTS

DIRECTIONS: When we choose a target part, we need de facto permission from all other parts in order to communicate with it. To find out who might object, we ask the client “How do you feel toward this part?” As we locate reactive parts we try to persuade them to separate (that is, unblend) and let us proceed with the target part. Some target parts are particularly extreme and unpopular and will evoke a big reaction. This is an exercise for getting to know unpopular parts.

Find a target part, write it down (use extra paper to draw it if you wish):

Notice how you feel toward this part and make a list of every feeling:

If you are doing this alone, embody each reactive part, allowing it to do (that is, direct your posture and your movements) and say what it wishes by way of introduction.

THEN ASK IT: “Why do you feel this way toward” (the target part)?

ONCE YOU UNDERSTAND, ASK: “Will you trust me to get to know (the target part) so I can help with this problem?”

IF THE ANSWER IS NO, ASK: “Are you willing to let me get to know you better?”

IF THE ANSWER IS YES, thank the part and move on to the next until you have permission from all of them.

RETURN TO THE TARGET PART AND ASK:

- “Did you watch me negotiate with those parts who react so strongly to you?”
- “What was that like for you?”
- “What do you want me – and those parts – to know about your job and how you are trying to help?”
- “What would happen if you stopped doing this job?”
- “If we could help that part would you need to keep doing this?”

Finally, set an intention to return and help with the underlying problem (the part) this protector has named.

Meditation

GOING INSIDE TO IDENTIFY PARTS

DIRECTIONS: Follow the prompts, and adjust as needed. You may want to write down or record what you notice.

- Settle into a comfortable position.
- Notice your back against the chair, your feet on the floor and your contact with the ground.
- Close your eyes and take a couple deep breaths if that feels comfortable. Notice.
- Focus your attention inside and notice any thoughts, feelings or sensations that show up.
 - You may notice physical sensations – some may be pleasant, others may be unpleasant.
 - You may notice one feeling or many feelings.
 - You may hear one thought or many thoughts competing with each other.
 - You may notice blankness or foggy inside. That's okay.
- You may also notice your mind wanting to distract you and shift your focus away from that sensation, feeling or thought.
- Be curious about whatever you notice.
 - What does it want you to know?
 - What is it holding for you?

- If you can, send it gratitude for showing up, even if what you have noticed is negative.
- Notice how the part responds to your gratitude.
- When you feel ready, come back to the room.

Notice how your energy shifts when you focus your attention internally for a few minutes. Are you calmer, more peaceful or perhaps more agitated?

THE 6Fs: THE STEPS WE USE TO HELP PROTECTIVE PARTS DIFFERENTIATE FROM THE SELF

The first three steps (find, focus, flesh out) involve helping parts to unblend.

1) **FIND** the part in, on or around the body.

- Who needs your attention right now?
- Where do you notice it?

2) **FOCUS** on it.

- Turn your attention inside.

3) **FLESH** it out.

- Can you see it?
 - If so, how does it look?
- If not, how do you experience it?
 - What is that like?
- How close are you to it?

4) How do you **FEEL** toward the part?

- This question is our Geiger Counter for Self-energy. Any answer that is not in the ballpark of the 8 C's means that a second part is influencing our thoughts.
 - We ask this second part if it is willing to relax so we can talk to the target part.
 - "If it is not willing to relax," we ask it what it needs us to know.
 - This process may lead us to a second (or third, fourth...) target part.
- Reactive parts often need to feel heard and validated. We stay with them until they are willing to let us get to know the target part.
- Once they agree, we ask the client, "How do you feel toward the (target) part now?"

5) **BeFRIEND** the part by finding out more about it.

- The fifth step involves learning about the target part and developing a friendly relationship. This builds relationships internally (Self to part) and externally (part to therapist).
 - “How did it get this job?”
 - “How effective is the job?”
 - “If it didn’t have to do this job, what would it rather do?”
 - “How old is it?”
 - “How old does it think you are?”
 - “What else does it want you to know?”

6) What does this part **FEAR**?

- “What does it want for you?”
- “What would happen if it stopped doing this job?”

This key question will reveal any lurking polarization.

“If I stop feeling anxious, I’m afraid the suicidal part will take over.” Or it will reveal the exile it protects.

“If I stop feeling anxious I’m afraid Jane will feel all alone and worthless.”

TO GET TO KNOW YOUR CLIENT’S PROTECTORS: USE EITHER INTERNAL COMMUNICATION OR DIRECT ACCESS

Our goal in this manual is to facilitate your ability to spot and talk with protective parts. In the next two sections we illustrate how to combine the 6F steps with the two main strategies we have for communicating with parts: “internal communication” (that is, communication between the client’s Self and the client’s parts, a.k.a. in-sight) and “direct access,” an essential method for those times when protectors do not feel safe enough to separate and allow internal communication. We try internal communication first because it brings the client’s Self in faster. But when internal communication is not possible because the client has no access to the Self internally, we use direct access instead. In both sections we offer annotated case examples, exercises for you to practice personally and with clients, and relevant neuroscience.

INTERNAL COMMUNICATION VS. DIRECT ACCESS

<u>INTERNAL COMMUNICATION</u>	<u>DIRECT ACCESS</u>	<u>A PROBLEM</u>
(a.k.a. in-sight)		
Client's parts	Client's parts	Client's parts
Client's Self		
Therapist's Self	Therapist's Self	Therapist's parts

Later in the manual we illustrate the “unburdening process,” which involves a series of steps that the client’s Self goes through with exiles. However if an exiled part arises at any point while you are looking for or talking with protectors, we strongly discourage you from trying to use IFS. Missteps with protectors can be repaired with a combination of genuine concern, interest in what happened and a willingness to apologize. Missteps with exiles, however, set the protective system on high alert and may cost the client much in time, energy and suffering. If you come upon vulnerable, wounded parts we urge you to proceed with your normal therapeutic approach until you have more formal training in the IFS model (see Self-Leadership.org for training opportunities).

DECONSTRUCTING THE 6Fs

Unblending in the First 3 Steps

In IFS we aim to convince protectors to unblend (separate) and allow the client’s Self to access and heal vulnerable parts. The following two cases illustrate how we do this by finding a target part, focusing on it and fleshing it out – all of which encourages the part to unblend (make room for the client’s Self) so that we can proceed with internal communication (the client’s Self talking to the part).

HELPING A PROTECTOR UNBLEND IN 3 STEPS

Alice's parents were unhappily married, self-involved and neglectful of her. Her working parents often left her and her younger sister home alone on various military bases. When she was 4 years old a friend of her father's stopped by when the children were alone and molested her in the bathroom of her own house. In addition, her hyper-religious, verbally and physically violent maternal grandmother lived with her family off and on throughout her childhood and terrorized her. Alice's principle protectors included a harsh critic, a bulimic and a dissociative part.

Find a Target Part

THERAPIST: How much of your mental time and space goes to the critic?

ALICE: Oh he gets most of it. Like 70 percent. And the fog gets the rest. The part who does stuff with food is not in my head so much – it just eats.

- *The client designates a gender for the critical part.*

Focus on the Part

THERAPIST: I'm guessing the critic has a strong influence on the fog and the part who eats. Do they agree?

ALICE: Yeah.

THERAPIST: Shall we talk to the critic first, then?

- *Asking for permission.*

ALICE: He's not fun.

THERAPIST: So other parts don't like him?

- *Acknowledging and being curious about internal relationships.*

ALICE: They're scared of him.

THERAPIST: That makes sense. Would they let you talk to him?

- *Asserting that the client has a Self (you).*

ALICE: Okay.

Flesh the Part Out

THERAPIST: Where do you notice the critic?

ALICE: In my throat.

- *Alice's critic, a manager, lives in her throat. The fog and binging parts distract her from its shaming criticisms. They are firefighters.*



Here is another example of Finding, Focusing On and Fleshing Out a protective part.

Find

THERAPIST: I hear you have a part who is angry with your girlfriend and another part who is afraid of losing her.

ENZO: Yes.

THERAPIST: Who needs your attention first?

ENZO: The angry one.

Focus

THERAPIST: Go inside and notice where the anger is located in, on or around your body.

ENZO: In my arms and hands.

Flesh Out

THERAPIST: What do you notice about that?

ENZO: He's like a boxer. He's in that *I'm ready* pose with gloves right up in front of my face.

- *Enzo finds his boxer protector, a firefighter who reacts to his girlfriend's criticisms, in his arms and hands.*



As these examples illustrate, finding a part, focusing on it and fleshing it out are all steps that foster unblending, without which internal communication between the client's Self and parts is not possible.

HELPING PARTS UNBLEND: SOME EXTERNALIZING TECHNIQUES

This topic could be elaborated on in its own book so the ideas we list below are not exhaustive. Externalizing helps parts to differentiate. While many clients are able to turn their attention inward and get enough separation from parts to communicate with them, some clients, especially those with trauma histories, are at first substantially blocked by protectors and will distract or dissociate if invited to pay attention to their internal experience. For them, externalizing options can be particularly useful and can be done with any one of a great number of props – invite your imagination and creativity.

Just as we differ in our use of externalizing props, we recommend that you play with these options to see what suits you. We also ask our clients about their creative interests and skills. Dancers may like to use movement, either literal or in the mind's eye; visual artists may wish to draw, paint or sculpt with putty or clay; fabric artists may want to sew or weave, etc.

And of course therapists who draw, paint, sculpt, dance or practice other forms of mindful movement may be particularly interested in (and good at) facilitating these activities with clients (whether they have training or not) who are open to trying something new (McConnell, 2013). Some IFS therapists will use a whiteboard, especially at the outset of therapy when the focus is on getting to know protectors. Illustrating inner relationships has several beneficial effects: as the client's complex internal world is externalized and made more tangible on the board, he notices, listens, ponders and connects. This exploration is a respectful enactment of curiosity, an open invitation to the client's system to be seen and understood. It also illustrates the ubiquity of

polarities and gives the therapist an opportunity to honor the balancing efforts of both sides in a conflict while asking some all-important questions: “Who does this one protect? What would happen if it stopped protecting in this way?” These questions reveal the exile who is being protected.

When protectors are very determined to distract, sand tray toys (which can be collected from yard sales and used with or without the sand tray) can help the client engage playfully. When a part is placed into a small figure and rendered as an external presence – a small one – its exaggerated features can be fearsome, funny or sad without being scary or overwhelming. Monsters become manageable and are invited to talk, babies of any species can be cuddled in the palm and jiggled on the knee.

EXTERNALIZING PARTS

Tom grew up in a low-income housing project. He was the youngest of three boys, his mother worked two jobs and his father, an alcoholic who worked for the postal service, came home only now and then. When he did, Tom's mother fought with him verbally and sometimes physically until he left again. In the rare times Tom encountered his father sober, he saw a cold, anxious man who would not touch him. In contrast, his drunken father, whom he saw most often, was funny and affectionate.

Meanwhile Tom's older siblings and other kids in the neighborhood picked on Tom because he was small for his age. When Tom came for therapy he was on disability, had been clean from heroin for four years and he dressed as if he were a member of a motorcycle gang.

Tom came to therapy because his psychiatrist asked him to. He often came late - frequently missing half the session. He felt overwhelmed by the alcohol counseling training program he was taking. And because he was struggling with urges to use, he was afraid if he went to AA or NA he would make connections to score drugs. His parts would not unblend so most sessions involved the therapist using direct access to speak to his parts. After three months he said he felt worse after sessions and wasn't sure that continuing was a good idea because the urge to use heroin just kept getting stronger.

THERAPIST: Thank you for letting me know. Can we go over all this? In sessions we talk to the part who wants you to go to a meditation center, the part who makes you sleep all day, the part who thinks you'll flunk classes and the part who eats a lot of junk food. They all think you're that lonely kid who struggles in school and gets teased by other kids. And they don't believe there's a Tom in there who has internal strength, not a part, right? Meanwhile the guy who uses heroin tells you he knows how to stop all this. That's a sure bet. He's done it before. I have a question for him. Would he be willing to try something new first? If he's willing, I want him to be the first to meet you, the Tom who's not a part.

- *Tom is silent, looking out the window for a few moments.*

TOM: Reluctantly.

- *The therapist pulls two smallish tables in front of his chair and places a tray of toy figures on the tables.*

Find, Focus and Flesh Out All in One

THERAPIST: Ask the heroin-using part to pick someone from here to represent him.

- *Tom looks at the figures carefully. They run the gamut from monsters to baby animals to kids and human babies. He picks a monster figure with its big mouth wide open and its arms looming in the air to intimidate.*

TOM: This is him.

THERAPIST: Put him on that table wherever he wants to be. How do you feel toward him?

TOM: He scares me. And I like him a lot.

THERAPIST: Let both of those parts pick figures to represent them, too.

- *Tom picks out two figures: a lamb and a tall female whose limbs are sewed on. She looks like a cross between Frankenstein and a zombie.*

THERAPIST (continued): Where do they want to be while you talk to him?

TOM: This one wants to be behind me.

- *He puts the lamb out of sight behind his back on the chair.*

TOM (continued): This one wants to be right next to him.

- *Tom places the female Frankenstein zombie next to the monster.*

Feel Toward

THERAPIST: Now how do you feel toward the heroin guy?

TOM: Sad. He's done a lot of damage but I know he was just trying to help.

THERAPIST: How does he respond?

TOM: He's puzzled. He doesn't know who I am.

Befriend

THERAPIST: He might be interested to know that you can help that part he protects.

TOM: He says I've done a crappy job so far.

THERAPIST: He might be interested in this, too. When a part takes you over you can't be there to help. For you to be able to help, everyone has to agree to give you room. Would he be willing to go first? If any other parts step in, we'll ask them to make room for you, too.

TOM: He wonders what your game is.

THERAPIST: Okay. That's a fair question. Let him know we're not trying to get rid of him. My goal is for all your parts to feel better, including him. He could quit all this and do something else - whatever he wanted. Would he like that?

TOM: He might.

THERAPIST: Would he let us show him what's possible?

TOM: He'll give it a shot as long as we know he's not going anywhere.

THERAPIST: Great. Do we have his permission to ask who needs your attention first?

- *Asking for permission.*

TOM: Yes.



By finding and focusing on his vulnerable part, an exile, as well as his very wary heroin using protector, a firefighter, Tom was able to help the latter part begin to differentiate, which helped him befriend it and get its permission to proceed.

As this vignette illustrates, externalizing parts encourages them to unblend and notice the client's Self. IFS therapists often find ways to suit themselves as they get creative with externalizing strategies. Other options include colorful scarves, pillows of various shapes and sizes, stuffed animals, or paper plates on which a part's identity can be written. When it comes to externalizing parts, we have the opportunity to be creative and invent new options.

Exercise

FIND, FOCUS ON AND FLESH OUT A TARGET PART

DIRECTIONS: This exercise walks you through the process of locating a target part. You may want to record the instructions below on a phone or other device so you can listen to them. Begin by turning your attention inside.

- Breathe and go slow.
- Remind your parts that there is room for everyone.
- Notice sensations, feelings and thoughts.

- Ask:

“Who needs my attention?”

Write that down:

- Continue to observe, be patient, and notice what shows up.
- Notice if any sensations, feelings or thoughts are being dismissed internally as insignificant or not real.

If so, start by being curious about the part who is trying to steer you this way. Write that down:

If not, choose whatever comes to mind first as your starting point. Notice where this part (sensation, feeling, thought) is located in, on or around your body.

- Do you see the part?
- Feel it?
- Hear it?
- Sense it in some other way?

Write down what you notice:

REMEMBER THE TARGET PART FROM THE WEEK BEFORE

Whether we are at the very beginning of a therapy or we are in midstream, we go through the steps of finding, focusing on and fleshing out target parts repeatedly. However, given the arbitrary time constraints of therapy sessions, once a therapy has been launched and we are in process week-to-week, we may not finish helping a particular part in one session so we return to it the next. Failing to do this can interrupt the flow of therapy and challenge the ability of protectors to develop trust. Consistency is as important in IFS therapy as it is in the lives of children.

We set the intention to return to a part jointly with the client. Since protectors who ward off pain often cause clients to forget the content of sessions, we can invite clients to record sessions and listen to them during the week. But either way the job of remembering at the start of the next session falls to us, so we make a note. If the client continually brings up a different part each week and objects to going back to the same target part, make a case for doing so anyway. If the client still insists on changing the subject, be curious about why and let the previous part know that we will come back to it later. The parts who are involved will notice. When we are reliable we win trust.

BEING CONSISTENT ABOUT GOING BACK TO A PART FROM THE PREVIOUS WEEK

THERAPIST: Is there anything we need to touch on before we go back to the part who felt so criticized by your grandmother?

MEGAN: Oh yeah. I forgot.

- *A protector.*

THERAPIST: Remember we set the intention to come back to her?

MEGAN: Did we? I'm feeling all that is pretty distant right now. I had an argument with Billy and I just don't know if I should be dating him again. That feels much more important right now.

- *A protector wants to change the subject.*

THERAPIST: I understand why that feels important. How about we split the time and check in on the 10-year-old who we were talking to last week first.

- *Negotiating.*

MEGAN: I don't think there will be enough time for everything. I really need to think about my relationship with Billy.

- *A protector.*

THERAPIST: I can understand that. But there is a problem. If you don't follow through on promises, your parts won't trust you. So I want to make the case for doing both.

- *Validating and making the case to this protector that it's important to let us go back to the part from last week.*

MEGAN: You know I'm just hearing "It's a bad idea!"

THERAPIST: Really? Can we ask why? What would happen if this part let you talk to the 10-year-old from last week now?

- *Checking on protector fears.*

MEGAN: I'll feel overwhelmed. I have a lot to do at work this week.

THERAPIST: If the 10-year-old agrees not to overwhelm you, could we check-in with her again?

- *Addressing that fear.*

MEGAN: Okay – only if she agrees.



Megan had been talking to an exile the week before (the 10-year-old). A protector who feared Megan would be overwhelmed by the feelings of this exile had taken over during the week and was reluctant to let that line of inquiry proceed.

Since the protector was concerned about emotional overwhelm and tried to prevent her from going back to the target part from the week before, the therapist had to be polite but persistent, exploring the resistance and addressing the concern. If she had not done so, the target part would have felt abandoned.

UPDATING PROTECTORS IN THE PROCESS OF UNBLENDING

We work to reverse the parentification of young parts. These parts, who have been inducted into an enduring sense of responsibility for exiled vulnerability in adult caretakers, may not be able to conceive of a responsible adult and will often be deeply distrustful of the client (and therapist) as adults at first. We therefore emphasize our intention to reverse the formerly upended order of care. With the aim of helping protectors let go of oppressive responsibilities and return to being children we often:

1. Ask protectors how old they think the client is (the answer is often in the single digits).
2. Guide the client to correct this notion by introducing them to the client's Self.
 - a. "Let the part know that you're grown up now. Can it see you? Offer to show it around your life."

3. Guide the client to repeatedly offer help and love to protectors as well as to the exiles they protect.

Overwhelming experiences that threaten the client's survival typically open a chasm between parts and the client's Self. Traumatized protectors commonly believe that the Self was rendered helpless during the trauma and is also stuck in the past. Introducing the client's "Self of today" shows the protector that the Self survived and is available to heal wounded exiles.

UPDATING A PART

CHARLEY: I hide food. It even seems crazy to me. My husband finds muffins and bagels in my bureau or my bedside table and he thinks it's pretty strange but he just laughs.

THERAPIST: Does that part know you have plenty of food now?

CHARLEY: My mom put a chain around the fridge and padlocked it. We were always hungry. My youngest brother and I used to go around the back of Stop & Shop at night to get the fruit and vegetables they threw out.

THERAPIST: Does the part who stashes food for you now know that things have changed?

CHARLEY: No.

THERAPIST: Ask how old it thinks you are and don't censor the answer, just say what comes up.

CHARLEY: Strangely, I hear 10 years old. That really surprises me.

THERAPIST: That's when your dad left, right?

CHARLEY: Yes. We really had nothing for a while there. But it was late summer so we kids found berries in the woods and stole from the farmer next door. When school started my mom got hospitalized and we were sent to foster homes.

THERAPIST: Let's give your food stashing part a chance to get to know the you who is not a part. Would he like a tour of your life today?

CHARLEY: He's amazed. He had no idea.

THERAPIST: What do you say to him about food?

CHARLEY: We're okay. That's never going to happen again. I'm showing him how our refrigerator opens and closes and we have a shopping list so he can pick whatever he wants.



Charley's food stashing protector still saw Charley as a starving 10-year-old boy, not an adult man with a husband and refrigerator of his own that is full of food.

As we see in this example, parts can be completely unaware of the present, in which case they need to be updated.

THE FIRST 3 STEPS: FIND, FOCUS AND FLESH OUT A PROTECTIVE PART

Find

- Ask:

“Which part needs your attention today?”

- Or, listen to the client talk.

- Repeat (or write on a whiteboard) the major themes you hear.

- Ask:

“Of these parts who needs your attention first?”

Focus

- Invite the client to go inside and notice where the part is located in, on or around the body.

- Invite the client to focus on that.

- Focusing on a part internally is different from talking about it.

Flesh Out

- Ask:

“How do you experience this part? Do you see it? Feel it? Hear it? Or is there some other way in which you are aware of it?”

- Ask:

“How old is this part?”

“How old does the part think you are?”

AFTER FIND, FOCUS AND FLESH OUT: A FORK IN THE ROAD

The 6F steps help us form the alliance we need with protectors. At this point, after the first three steps and before the last three, we come to a fork in the road: if the client's protectors have been willing to unblend, you will continue through the last steps using internal communication. But if you are not able to convince protectors to separate – which is often the case with trauma – you have to give up (for the moment) and move to "direct access" (the therapist's Self talking directly to the client's part). But before describing direct access in detail (starting on p. 102), we will introduce some neuroscience concepts that are relevant to IFS and finish illustrating how to work with protective parts using the technique called internal communication.

INTRODUCTION TO NEUROSCIENCE

The “decade of the brain” met the world of psychotherapy several years ago now, with leaders in the field of neuroscience emerging to help us better understand what is happening in the brain during psychotherapy and how we can effectively help people heal from the sequelae of traumatic experiences. Knowledge of neuroscience can also inform therapeutic decision-making. For example, under what circumstances does it make most sense for the therapist to remain calm and non-reactive? And when is it best to be confident and speak up? In addition, when does it make sense to slow down and work with the body? In this manual, we incorporate what we believe is happening in the brain as it relates to the IFS model of therapy.

SCIENCE: THE MIND–BRAIN RELATIONSHIP

Many mental health theorists and practitioners distinguish between mind and brain in the following way: as the mind deals with energy and the flow of information, it primarily relates to *functioning*, while the brain relates to *structure*, defined as a collection of interconnected neurons, networks and neurotransmitters in the head, both of which interact with the body and the environment (Siegel, 2017).

As therapists we deal with both: the mind when our clients notice and interact with their thoughts, feeling and sensations, and the brain when internal attention allows for therapeutic change which coincides with structural changes in the brain (neuroplasticity). Some scientists also believe that the mind is capable of changing states, rapidly moving from one cluster of mental activity (or collection of neural networks) to the next, each serving a specific function

(Siegel, 2017). This view is consistent with the most basic premise of IFS: that the psyche (mind) is naturally comprised of different parts (mental states) that are capable of blending or stepping back (shifting states).

We believe that parts use both the brain and the mind, primarily living in the mind and utilizing the brain. Parts have a range of thoughts, feelings and sensations, and they use the neural networks that correspond with those specific thoughts, feelings and sensations in the brain to express themselves.

THE SCIENCE OF FIND: IDENTIFYING PARTS

Groups of nerve cells (neurons) come together to form neuronal circuits or networks. Neuroscientists are continually discovering, mapping out and identifying various brain networks, which run the gamut from the brain at rest to empathy, compassion, grief, care, seeking and panic along with those networks that have been affected by traumatic experience.

In IFS terms, the process of finding a target part involves the mind focusing energy and attention on a specific cluster of neural networks that serve a specific function.

THE SCIENCE OF FOCUS: GOING INSIDE AND MEDITATION

When we talk about focus we are generally describing how we pay attention. When we are externally engaged – focusing outside ourselves on our environment or our relationships – we are utilizing *exteroceptive awareness* (Seppala, 2012), which primarily relies on the prefrontal cortex. *Interoceptive awareness* (Seppala, 2012), on the other hand, involves internal focus and relies on deeper brain structures such as: the brainstem (linked to physical sensations like heart rate and breathing), the limbic system (emotional integration), the insula (body awareness) and the posterior cingulate (connected to self-awareness) – all of which are affected by trauma.

Talk therapy typically utilizes exteroceptive awareness, with clients focusing their attention on dialogue with the therapist. In contrast, in IFS we invite our clients to go inside and focus internally (interoceptively) on the relationship between themselves and their parts. Interoceptive awareness is said to have more power to influence our level of happiness (Seppala, 2012).

There are of course many different forms of meditation. Mindfulness, described by Kabat-Zinn as a nonjudgmental awareness of the present moment (Kabat-Zinn, 2003) is one popular form of meditation that has been studied and shown to have both physical and mental health benefits. Regarding trauma, meditation is known to have a positive effect on several brain structures that are adversely affected in people who live through overwhelming experiences. In IFS, meditation aids clients with the process of unblending from parts and accessing Self-energy. Mindful separation, or the ability to unblend and *be with* rather than *being in* an experience is a prerequisite for the final steps of healing in IFS because reexperiencing (reliving without a validating inner audience) is not therapeutic.

THE SCIENCE OF FLESH IT OUT: CLARIFYING THE PART

Once the client has identified the target part and begins to focus his attention internally we guide him to engage with the part. This generally occurs with the mind's eye but for those who are not visual the experience can be sensory, kinesthetic or aural.

- Where is the part located in or around the body?
- Does he see it? Feel it?
- Or perhaps hear it?
- Does the part have a shape?
- A color?
- A size?
- A sound?
- How old is it?
- How close is he to it?

All of these questions help the therapist and the client to flesh out and better identify the part.

In IFS many protective parts (otherwise known as symptoms) are rooted in fear. They tend to overwork and cause upset. We believe these parts live in the mind and express themselves by connecting with unintegrated (or dysregulated) neural circuits in the brain, which typically over- or under-function and result in psychic pain. Once the wounded (exiled) part is healed, protective parts are able to let go of their job, function normally (as they did before the wounding occurred) and structurally integrate back into the larger system.

Exercise

GET TO KNOW A PART

This meditation is designed to help you get to know a little bit about a part that you want to help or change your relationship with.

- If it feels good, go ahead and take a deep breath.
- *And think of a part who you'd like get to know a little better.*
- *Go ahead and focus on that part wherever you find it, in, on or around your body. — If you can't focus on it, that's okay.*
- Either way, notice how you feel toward it.
- *If you feel anything other than curiosity or acceptance, ask the reactive part if it would be willing to separate from you and not interfere just so you can learn more about your target part. We're not going to let it take over, we're just going to get to know it.*
- *And keep doing that with reactive parts until you feel curious about the original part.*
- *You may find that you don't get there, that other parts won't separate, which is okay. You can just spend the time listening to their fears about separating.*
- *But if they do let you feel at least curious about the original one then it's safe to listen.*
- *What does that original part want you to know about itself?*
- *What has the part been trying to do for you? To you?*

- *What might it need from you?*

I'll stop talking for a little while now and let you get to know it and then come back with time to return.

- *Okay, in the next few minutes we'll begin to come back.*
- *Thank the part for letting you know about it.*
- *And let it know this doesn't have to be its only chance to talk to you. If it wants, you can come back to it another time.*
- *And before you come back to this room, make sure you thank all the other parts for letting you get to know this one or letting you know that they were afraid if they didn't.*
- *And when all that feels complete, you can, if it feels right, begin to take some deep breaths again and shift your focus back to the outside.*

THE 4TH STEP: ASSESSING SELF-ENERGY WITH FEEL TOWARD

Our overall goal is for all parts – the target part and any reactive secondary parts – to differentiate and make room for the client's Self to heal the wounded part. The 4th step contributes to this goal by helping us check on the client's level of access to the Self. The question "How do you feel toward this (the target) part?" is our Geiger Counter for Self-energy (and, conversely, for blended parts).

As we assess the client's inner relational milieu in this way we may hear that someone is scared of the target part, that someone is afraid the target part will overwhelm or it's so crowded inside that there is no room for the Self. By checking on the internal scene in this way, we locate any parts who need reassurance.

Here are two examples of using the 4th step—along with two exercises for accessing Self-energy.

4 STEP: FEEL TOWARD

Feel Toward

THERAPIST: How do you feel toward the boxer part?

ENZO: He protects me! I like that.

THERAPIST: How does he respond?

- *The protector's response will tell the therapist whether Enzo's appreciation is from the Self or another part.*

ENZO: He's used to ignoring me.

- *Enzo's appreciation is from a part – the boxer would not be ignoring the client's Self.*

THERAPIST: Because?

ENZO: He thinks I'm weak.

Flesh Out

THERAPIST: How old does he think you are?

ENZO: Oh... a little kid.

THERAPIST: So the little kid appreciates the boxer? And the boxer thinks the little kid is weak. Would the little kid be willing to separate so you can talk to the boxer?

- *We ask secondary parts – in this case the little kid – to differentiate so the client's Self can be present and talk to the target part. Our goal is to get the client's Self in the middle between this protector and this exile.*

ENZO: He says okay.

Feel Toward

THERAPIST: How do you feel toward the boxer now?

- *Assessing the client's Self-energy again.*

ENZO: He turned around to look at me for the first time. He's surprised. He doesn't know me. I'm thanking him for all his help.

- *When the little kid separates, the protector notices the client's Self.*



In this example we see two parts, a protector and the exile it protects, becoming aware of the client's Self for the first time simply because the therapist's Self is guiding them and holding the space for this to happen.

Exercise

FEEL TOWARD

DIRECTIONS: Locate a target part, focus on and flesh it out, and then ask:

“How do I feel toward _____ [the target part]?”

Write the answer down:

If the answer is one of the feelings (or some simile) in the list below that indicates Self-energy, move on to befriending the part.

FEELINGS THAT INDICATE THE CLIENT HAS SELF-ENERGY FOR THE TARGET PART:

- Curious
- Open
- Kindly
- Caring
- Connected
- Concerned
- Compassionate
- Loving

If, however, the answer is “I understand,” you need to notice whether you are hearing from a manager who forestalls feelings by staying in thoughts and telling a story that sounds plausible; or if you really have a heartfelt connection with the part and you are aware how it feels.

If you are not sure where this “I understand” comes from, tell the target part what you understand and ask if you’ve got it right.

If, on the other hand, you hear “I agree with this part” then the part is not yet differentiated enough from you to have a conversation. Ask it to separate so you can talk.

And if the answer is any other feeling (for example, hatred, anger, fear, embarrassment) ask the part who feels that way: “What are you concerned would happen if you were to relax and let me talk to _____(the target part)?”

Reactive parts commonly fear the target part in some way. Write down whatever you hear:

If the reactive part fears the target part gaining too much influence, ask:

“If _____(the target part) would agree not to take over, would you let me talk to it?”

If the reactive part replies:

“That will never work,” you can reassure it in two ways, first by asking, “Can we hear from _____ (the target part) directly about whether it’s willing to not take over?” And then by saying to the reactive part: “You’re the boss. I’m not trying to make you do anything you’re not comfortable with. But if you let me hear from both sides, I can help with this conflict between you and _____ (the target part.)”



ASSESSING THE CLIENT'S LEVEL OF SELF-ENERGY IN THE 4TH STEP

Feeling Toward

Polly has noticed a critical part, and described it as a little man in a business suit.

THERAPIST: How do you feel toward this critic?

- *In response to this question we listen for the reactivity of other parts in her answer.*

POLLY: I don't like him.

THERAPIST: Would that part be willing to let you handle this?

- *Asking permission again.*

POLLY: I doubt it.

- *The reactive part is not inclined to cooperate and let the client talk to the target part (the little man in the business suit). This tells the therapist that the reactive part believes the target part is a hazard.*

Fleshing Out

THERAPIST: So you have a part who dislikes the critic and doesn't trust him. Can I ask a question? Who does this distrustful part think you are?

- *The client's answer to this question will reveal the vulnerable part.*

POLLY: A kid.

Enough Self-Energy?

THERAPIST: What do you say to it?

- *Assessing how blended the vulnerable part is with Polly - if it is blended then Polly will say she does feel like a kid.*

POLLY: I'm not a kid. I'd actually like to talk to the critic. I do sometimes wonder why he goes on and on like that.

- *Polly's Self is present.*

THERAPIST: See if everyone inside will let you ask the critic why he goes on and on like this.

- *Asking all worried parts to relax and let Polly's Self talk with the critic.*



In this example, we discover Polly's Self is present simply by noting how Polly responds to the protector who believes she is still a child.

Exercise

ASSESS SELF-ENERGY: FEEL TOWARD

DIRECTIONS: For the sake of clarity, the target part in this exercise will be a chronically anxious part – but you can write in the part you hear about internally.

ASK: “How do you feel toward the _____ [anxious] part?”

- If the client is significantly differentiated from reactive parts and says something along the lines of “I’m curious” or “I care” then proceed and be curious: “What does this part want to tell you about itself?”
- But if the client has a negative reaction like “I hate it!”
- Or, the client expresses agreement along the lines “Of course I feel anxious.”

ASK: “Does this _____ (hating or agreeing) part need your attention first or would it be willing to relax and let you be curious about the _____ [anxious] part?”

- If the reactive part is willing to relax, repeat the original question: “How do you feel toward the _____ [anxious] part now?”
- If, however, the client goes on agreeing with the anxious part, we guide the client to inquire whether the anxious part is blended.

To do this, simply ask: “Is the _____ (anxious) part blended with you right now?”

- If yes: “Would it be willing to separate and meet you?” (meaning the client’s Self)
- If no then we explore more to find out who is agreeing: “Okay, somebody else agrees and maybe that part wants to say something. Let’s ask all the parts who agree on this issue to meet with you in a group. Would they be willing to do that? Invite them all to join you at a big conference table and see who shows up.”

After choosing a target part, the goal is to persuade reactive parts to unblend (differentiate), which will allow the client’s Self to be present and befriend the target part. When a part reacts negatively (or positively) to the target part and will not unblend, we move on to the method called “direct access.”

A CAVEAT: BE AWARE OF SELF-LIKE PARTS IN THE 4TH STEP (FEEL TOWARD)

Self-like parts present with expressions of feeling that sound like the Self (“I care, I want to help”) and they often stand in for the Self in the mind’s eye, but they are protective parts, which means they are not equipped to heal wounds. An exiled part may feel very attached to a Self-like part who has been comforting it over the years or it may feel smothered and resentful. In either case, when we mistake a Self-like part and proceed as if it is the Self, progress often moves too quickly and easily, or halts altogether and the exile can seem perversely uncooperative. This is a sign that a Self-like part is at work and needs help to relax and trust the client’s Self. Self-like parts can also show up when the client’s parts are trying to please the therapist and are overly compliant. Self-like parts are common in trauma survivors because sensing the needs of others and giving them what they want can be protective.

SPOTTING A SELF-LIKE PART: WHEN PROGRESS IS TOO FAST AND EASY

Find

THERAPIST: Would your depressed part be willing to separate so you can get to know it better?

BRENT: Yes, it will.

THERAPIST: How do you feel toward it now?

BRENT: I feel fine about it.

THERAPIST: Would the part who feels fine also step back?

BRENT: Sure.

THERAPIST: How do you feel toward the depressed part now?

BRENT: I feel compassionate.

THERAPIST: Is it taking that in?

BRENT: Sure.

- *Brent seems disconnected. The therapist senses he has no traction internally.*

Find and Focus

THERAPIST: I'm wondering if you have a part who is trying to help move things along?

BRENT: What do you mean?

THERAPIST: Is someone trying to help? Take a moment to ask inside.

BRENT: Actually there is. How did you know? There's a part who wants to help you and me so it says what it thinks you want to hear.

THERAPIST: Thank it. And ask if it would be willing take a break and watch us. We'll be okay whatever comes up, even hard things.

- *Asking for permission.*

BRENT: I'll ask.

- *He closes his eyes and is silent for several seconds.*

BRENT (continuing): I think this part is around a lot. It wants to do the right thing. It's been with me a long time.

Feel Toward

THERAPIST: I bet it's been helpful. How do you feel toward it?

BRENT: I appreciate it.

Befriend

THERAPIST: Let it know.

BRENT: It likes that.

THERAPIST: Is it willing to trust you?

- *Will this part unblend? Is it willing to notice the client's Self?*

BRENT: It's not so sure about that.

THERAPIST: How old does it think you are?

BRENT: Fifteen.

THERAPIST: What do you say to that?

- *Inviting the client's Self to take the lead.*

BRENT: The 15-year-old is depressed. This part is confused and thinks I'm him.

- *The Self is present.*

THERAPIST: Does it see you now?

- *Still befriending.*

BRENT: Yeah. It's kinda shocked.

THERAPIST: Would it let you help the 15-year-old?

- *Asking for permission.*

BRENT: It's confused by all this. It's always taken care of me.

- *Brent is describing a protective part in the Self-like role – that is, this protector stands in for Brent's Self and tries to handle his life and take care of younger parts.*

Flesh Out

THERAPIST: It stands in for you?

BRENT: It sorta *is* me.

- *Self-like parts often insist: I am the client.*

THERAPIST: I hear it has taken the lead because it had to and wasn't aware of the Brent who's not a part. What's it like for this hard working part to meet you?

- *Asserting that the client has a Self.*

Protector Fears

BRENT: It's really surprised. What will happen to it?

THERAPIST: It won't go away. It will still be part of you.

- *Reassuring the protector.*

BRENT: That's a relief.

THERAPIST: It's worked so hard. Will it accept your help?

BRENT: It's thinking. It is tired.

- *Self-like parts often need a lot of reassurance before they are willing to turn the keys over to the Self. Don't worry – be persistent.*



If you feel a puzzling lack of traction in a session or things move too easily, suspect a Self-like part. As this example illustrates, one of the ways Self-like parts manifest is by making the client seem “cooperative” when he is essentially absent.

SCIENCE: ACCESSING THE SELF WITH FEEL TOWARD

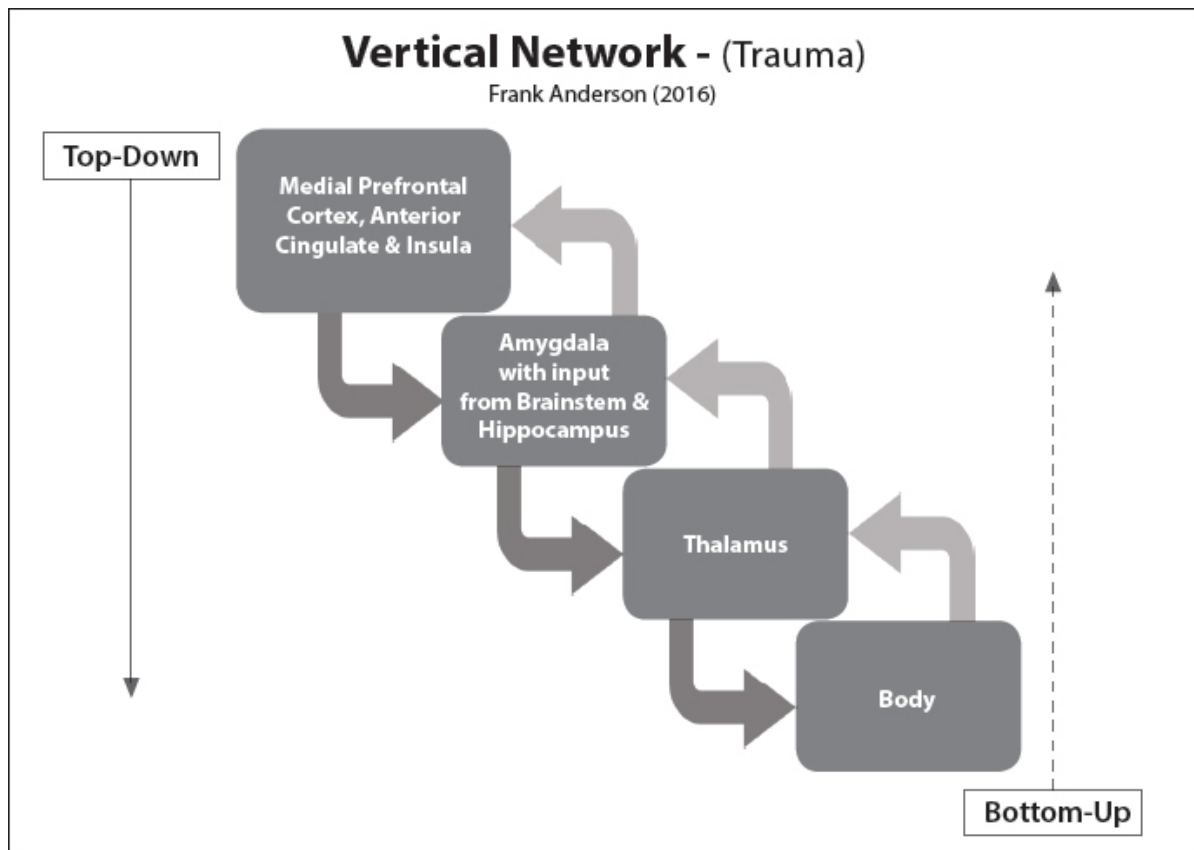
Neuroscience is exploring brain structures that correlate with self-definition, including how we represent, evaluate and monitor ourselves. These activities are connected, respectively, to the medial prefrontal cortex, the dorsal lateral prefrontal cortex and the anterior cingulate (Nortoff & Bermpohl, 2004). We know that self-identity also involves connections to our body and physical sensations through a structure called the insula (Lanius, 2010). Although these advances in neuroscience help us understand how the brain is involved in contextual self-awareness they are different from the IFS concept of the Self.

The Self of IFS, involving a state of calm, curiosity, confidence, courageousness, clarity, connectedness, compassion and creativity, is not, in our view, located in any specific brain structure. We believe that the Self, which is *a state of being*, is located within the mind just like protective parts – but there is a difference. Protectors who developed in response to either small or capital T trauma and show up as symptoms use unintegrated networks in the brain. In contrast, **the Self accesses integrated neural networks and connects to the external world spontaneously. From our perspective, the experience of “being in Self” (as we say in IFS) is internally and externally connected and maximally integrative.** In short, the Self possesses inherent wisdom and the capacity for healing. In our view, once liberated from extreme roles protectors revert to using integrated neural networks as well.

SCIENCE: THE NEUROBIOLOGY OF TRAUMA AND DISSOCIATION

Therapy with clients who have experienced severe trauma is often challenging, time-consuming and at times overwhelming. Survivors often have parts who are dominated by implicit memory

(unconscious, tenacious, constituted mostly of feelings and sensations with no cognitive input or time sequence). Boiled down to its simplest denominator, the goal of trauma therapy is to convert implicit memory to explicit memory (conscious, factual, linear, with a sense of time and a narrative). The job of all therapists is to help traumatized clients transform raw, undigested memory into a cohesive story that includes feelings and beliefs as well as a beginning, middle and end.



Under normal circumstances when we experience something stressful, the nervous system takes the input from the body and carries it up through various brain structures, including the thalamus (sensory input), brainstem (heart rate and breathing), amygdala (emotional significance), and hippocampus (cognitive input), before going to the anterior cingulate, insula and prefrontal cortex, where the information is processed and the appropriate response – usually calming down – is determined (van der Kolk, 2014).

But when the individual's perception of danger increases because stress is more extreme, chemicals, including cortisol and adrenaline, are released to mobilize the body, moving us from a state of safety to a state in which we are ready for fight or flight (sympathetic activation). In

this scenario, the ability to process information and downregulate goes off-line, resulting in a state of high physical and emotional reactivity along with a limited ability to calm down and recover (van der Kolk, 2014).

If we continue to be trapped in a traumatic situation and feel helpless to escape, we move from sympathetic activation to parasympathetic withdrawal and eventual collapse. Here several structures in the brain go offline and internal organs slow down to conserve energy to increase the chance of surviving danger. In this state, we are disconnected from our emotions, our body and our ability to process information (Porges, 2011).

The Three States of Processing Stress

Normal Stress	Sympathetic Hyperarousal	Parasympathetic Blunting
Normal inhibition	Under-inhibition*	Over-inhibition*
Normal thoughts	Little cognitive perspective	Low thinking
Normal emotions	High emotions	Low emotion
Normal body sensations	High body sensations	Low body sensations

* (Lanius et al., 2010)

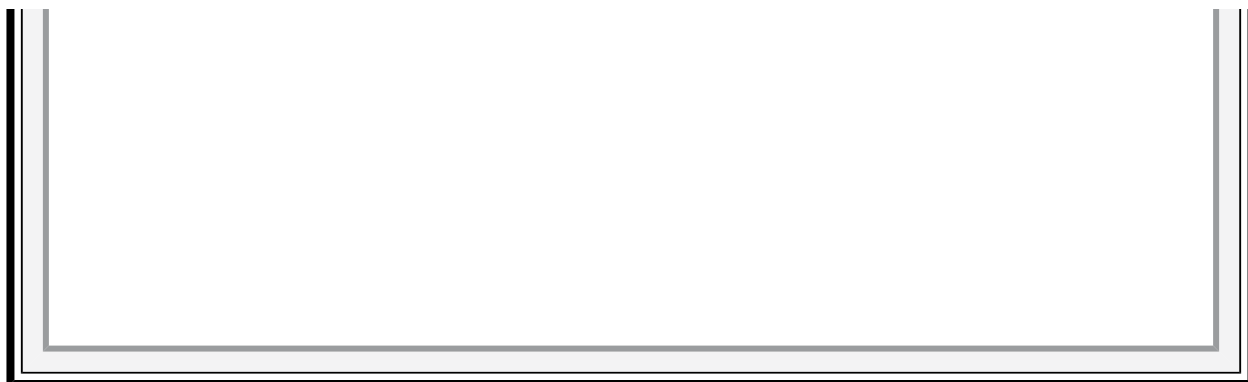
There are three states that therapists deal with when working with individuals who go through stressful situations. At times, a client is able to process information cognitively, emotionally and physically. At other times (of sympathetic activation) the client is in high emotional and physical activation with little ability to downregulate. At yet other times (parasympathetic blunting) she is numb, dissociated and without access to physical sensations, emotions or thoughts. In the most intense moments, a traumatized client may alternate among all three of these states.

Meditation

BE WITH YOUR HEART

- *If it feels good, go ahead and take a deep breath and notice your heart in whatever way you experience it – this doesn't have to be your actual heart on the left side of your chest, just however you experience your heart.*
- *I'm going to invite you to get to know it in a physical way and we're going to explore different qualities of your heart.*
- *We'll see about its condition.*
 - *First notice how open it is.*
 - *How tender, encrusted or callused.*
 - *Perhaps it is congested, fluid or flowing.*
- *Also notice how much space your heart has in there.*
 - *Does it feel contracted and tightly packed or spacious?*
- *You might find in your exploration that different places in your heart are different in those ways.*
 - *Maybe the front is closed but the back is open.*
 - *Or the top is tender and the bottom is tough.*
 - *Maybe energy can flow through some parts of your heart but not all.*
 - *Maybe in some places your heart feels pinched and contracted while in other places it feels spacious.*
- *So where it feels extreme in any of those ways – closed, contracted, contested or callused – you have located some protective parts.*
- *If you want, you can take a little time right now to meet those parts.*

- *If it's okay, just get a little curious about what those protectors are afraid would happen if they allowed you to open your heart fully.*
 - *If they let your heart be tender.*
 - *If they didn't try to contract or compact it.*
- *In answer to that question, you are likely to learn about the vulnerable parts they protect who also live in or around your heart.*
 - *Right now you don't have to go to those vulnerable parts, you can just hear a little bit about them from these protectors.*
- *As you get to know how vigilantly these protectors have guarded your vulnerability, just extend your loving appreciation toward them at whatever degree feels sincere.*
- *Notice how they react to that appreciation.*
- *Right now we are not asking them to change anything. Nor do we expect them to change.*
 - *We're just getting to know their fears and showing them appreciation.*
 - *And someday, if they want, if they feel it's a good idea, they may let you go to those vulnerable parts to heal them.*
 - *Then these hard working protectors will be able to relax and allow you to open your heart.*
- *Often protective parts don't believe healing is possible. They feel condemned to do this with your heart for the rest of your life. So just be sure they know it is possible but there's no pressure.*
- *When this visit with your heart protectors feels complete, you can begin to shift your focus back outside but make sure before you leave to thank them for letting you know all this and thank them for their hard work to keep your heart safe.*



THE LAST TWO STEPS IN THE 6F’S: BEFRIENDING AND EXPLORING PROTECTOR FEARS

The last two of the 6F steps (befriending and exploring a protector’s fears) are the way we get to know protectors, understand their motives and concerns (especially about exile overwhelm) and win their permission to help the wounded parts they protect. Finally, in this segment we look at the science of befriending in relation to empathy and compassion and of protector fears in relation to the neurobiology of trauma.

THE 5TH STEP: BEFRIENDING

Once we have found and focused on a target part, fleshed it out and located enough Self-energy in the client (“how do you feel toward...”), we have the opportunity to learn more about the part and foster the relationship between it and the client’s Self:

- “What is your job?”
- “How did you get it?”
- “How old are you?”
- “How old do you think the client is?”
- “Who do you protect?”
- “What do you want the client to know?”
- “If you didn’t have to do this job, what would you rather do?”

ASKING PROTECTORS ABOUT THEIR MOTIVES

We guide clients to ask protectors about their motives and responsibilities. And we support clients in listening to protectors with respect, interest and an open heart. New practitioners of IFS may have parts who feel daunted by a seemingly unending supply of parts in some clients, however this is not a true obstacle. Parts are like Russian dolls: parts can have parts, who have parts. But we do not ask clients to name all their parts nor do we guide them to have their parts notice subparts. If the conversation goes that way, however, our methods don't change. We focus on the parts who show up – that beehive of protectors revolving around an exile. In this way we are guided by the client's system and we need only learn as much as the system needs us to know to facilitate healing.

BEFRIENDING LEADS TO MORE UNBLENDING

Relationship building is our job throughout IFS therapy. We want parts to be in relationship with the client's Self, which becomes possible when parts make room for the Self inside. As the client befriends protective parts, we concentrate on persuading them to differentiate so the client's Self can be present and embodied.

LISTENING INSIDE

To befriend the client's parts, we focus on building the relationship between them and the client's Self by making sure the client is available to listen and the part feels validated. First and foremost, protective parts need to know that the client understands why they need to do their jobs. We therapists model the attitude that we are suggesting the client cultivate internally: respect for the efforts of protectors, the assumption that their intentions are positive and curiosity about their fears. We are non-judgmental and kind. The premier evidence of the Self (that purely differentiated state) is love, compassion (concern for another's suffering) and the desire to help. We look for this in ourselves and in our clients.

BEFRIENDING A PROTECTOR

Find

THERAPIST: I notice a part who overreacts at times. Are you aware of that part?

NAOMI: That's just me. I've been that way for years.

- *The overreacting part is blended with Naomi.*

THERAPIST: I know it feels like that but would it be okay to get curious and ask if this is a part who somehow protects you?

- *Validating and persisting.*

NAOMI: It would be strange to think of this as just some part of me.

- *The idea that a highly reactive part can unblend is a new concept for her system.*

THERAPIST: I know. Can we ask if it's willing to separate from you a little so we can get to know more about it?

- *Validating and persisting.*

NAOMI: I don't know how to do that.

- *Again, unblending is a new concept.*

THERAPIST: I can help. Think of a situation recently where you overreacted.

- *Reassuring and guiding.*

NAOMI: Okay. Last week I found out that my agency wants me to apply for the position of district manager and I freaked out.

Focus

THERAPIST: Great, that's exactly what I am talking about. Now even as you focus on that part see if you can get a little distance from it and ask it to tell you about the reaction.

NAOMI: It's funny but I keep hearing "You need to be prepared! Always be prepared!"

- *She has tuned in – or, as we say in IFS, “gone inside” – and listened.*

THERAPIST: Do you know what that's about?

- *Checking to see if she has a general awareness of what motivates this reactive part.*

NAOMI: Not really.

Feel Toward

THERAPIST: Want to find out?

- *Checking to be sure she has enough Self-energy toward the part.*

NAOMI: Definitely because I don't know where that came from.

- *She does have enough Self-energy.*

Befriend

THERAPIST: Great. Let the part know that you're curious and see what it wants you to know.

NAOMI: It doesn't like surprises. Surprises are bad. Really bad!

- *The first direct communication from this protector.*

THERAPIST: Ask it to say more.

NAOMI: I'm seeing times when my parents, mostly my mother, would scream at me out of the blue when I got something wrong. I was terrified.

THERAPIST: I bet you were. It sounds like this part tries really hard to be sure you're prepared so you won't be surprised like that again.

NAOMI: Yes exactly. I over-prepare with most things and I guess I overreact too when I don't feel prepared. I didn't know this was about her yelling at me all the time.

- *Rather than being the part (blended), Naomi is now in relationship with the part and therefore able to understand its reasons for reacting strongly to surprises.*

THERAPIST: Let the over-preparing, overreacting part know that we get it and we appreciate how it tries to protect you.

NAOMI: It likes that. It's not used to being appreciated.



As we emphasize throughout this manual, exile overwhelm is the number one fear of protectors. Luckily it is not hard to address.

As this example shows, protective parts can be completely blended. We can say this part is “in the driver’s seat” or that the client is seeing the world through its eyes. Helping parts to unblend sets the stage for all other progress in IFS therapy.

BEFRIENDING PROTECTORS

DIRECTIONS: When it's time to listen to inner protectors, be sure that you are available.

Scan internally while asking yourself:

- “How do I feel toward this part of _____ (the client)?”

If the answer is anything along the spectrum from curiosity to compassion, then it's okay to proceed.

But if the answer is anything else, then you have detected a reactive part in yourself. Ask the part:

- “What do you need from me in order to let me listen without judging?” (or being afraid, or whatever reaction you have noticed)
- When a reactive part shows up, validate its experience first.
 - “Of course it makes sense that you feel anxious about _____ [the client] confronting his boss. Would you be willing to dial the anxiety down so we can learn more about your concerns?”

THE SCIENCE OF BEFRIENDING: EMPATHY VS. COMPASSION

Once separation occurs between the target part and the Self, we move to befriending, which we can also think of as internal attachment work. We help the client discover who the part is and what it needs to share: What is its job? How old is it? Who is it protecting?

The mapping of neural networks related to compassion and empathy by neuroscientists helps us understand two distinct ways in which therapists can be with clients: Empathy involves our

capacity *to feel with* another person while compassion involves *feeling for* another person and includes the desire to help. While empathy can lead to burnout, compassion leads to resilience (Singer and Klimecki, 2014).

In IFS we view empathy as a (more or less intensely) blended experience in which the therapist or the client's Self feels what the part is feeling. Compassion, in contrast, is an unblended experience with the Self of both client and therapist present with perspective and patience. In the therapeutic journey of establishing trust and connection between parts and Self, parts often need to experience both ways of being in relationship. Together, being felt (empathy) and being helped (compassion) gives them enough comfort and safety to reveal their vulnerability.

THE 6TH STEP: ASSESSING PROTECTOR FEARS

Working with protectors can be challenging because of their reactivity and the potential that they will evoke exiled feelings and activate other protective parts in us or in the client. We need to stay self-aware, be honest about our parts, and tend to those who get activated to minimize their impact on the treatment. Protectors also resist accepting help for any number of reasons and their concerns often require the bulk of our time in IFS therapy. Here is a list of common fears and how we respond.

1. The part will disappear if it isn't needed in this job.
 - We assure the part: "You will not disappear. You are a part of Lucy now and you always will be a part of Lucy. Healing this injury will free you."
2. The therapy will end and the relationship with the therapist will be lost if protectors allow the client's Self to show up.
 - We tell the part: "There is room for Lucy's Self and for me, too."
3. A secret will be revealed.
 - We explore the dangers of this secret, addressing any false beliefs about consequences.
4. The client will be overwhelmed with pain.
 - We assert with confidence that the client's Self has the capacity to handle the part's intense emotions.
5. The therapist won't be able to handle the exiled pain.

- We stress that this is not true of the therapist's Self and that we will identify and tend to any of our parts who are reactive throughout the therapy.
6. A polarized protector will take over if this part relaxes.
 - We offer to negotiate this with the other protector if it does show up, always getting permission from all parts before proceeding.
 7. Self-energy is dangerous and will attract punishment.
 - We explore early experiences of Self-energy being punished, and we introduce parts to the Self slowly so they can experience it at their own pace and over time without bad consequences.
 8. There is no Self.
 - We assert that the Self was not destroyed and if parts will relax, it will naturally emerge.
 9. The therapist – or other parts – will judge this protector for the damage it has done.
 - We show compassion and assure the part that we will not judge and if critical parts do show up we will address them directly.
 10. Change will destabilize the client's inner system.
 - We explore the part's fears and beliefs about change and we assert that the Self will help the system stabilize.

EXPLORING FEARS

Sometimes a protector spontaneously speaks of its fears: "If I let him (the client's Self) in, what will happen to me?" But if not, we make a point of asking about protectors' fears for two reasons.

- First, the fear will point us either toward a polarized protector or it will reveal the exile – and we want that information.
- Second, fearful protectors need help. Until protectors are on board we won't have access to the client's Self or to exiles. So we ask protectors about their fears.

"What would happen if you were to stop doing this job?" There is much to be learned from the part's answer, particularly:

1. If a protector will not relax because it fears emotional overwhelm, we offer to ask the overwhelming part (an exile) to not overwhelm.
 - “I hear that you are concerned about Sean (the client) feeling overwhelmed. That makes sense. We don’t want that either. With your permission we can ask that part not to overwhelm.”
2. If this protector fears that a polarized protector will get too much influence over the client, we suggest a parlay to get the client’s Self between warring parts (Krause, Rosenberg & Sweezy, 2016)
 - “If the part who you’re worried about is willing to sit down with you and Sean instead of taking over, would you be willing too?”
 - “Sean, can you hear how each of these parts is trying to help you? Let them know that you value their input and ask if they’re willing to trust you enough to make room so all three of you can talk.”
3. Finally, if an exiled part is overwhelming the client, the therapist can talk to it directly.
 - “I’m really glad you’re here. I see that you’re afraid. Would you be willing to make just a little room so Sean can be here, too? If you let him come in with you, he’ll help.”
 - Note: This is where we urge you to revert to your expertise. Do not try to use IFS with exiles unless you have IFS training.

THE #1 FEAR: EXILE OVERWHELM

The number one concern of protectors is fear of emotional overwhelm. They’ve seen it happen and they’ve seen it debilitate the client, often with crippling depressions, anxiety or repeated hospitalizations. We validate their fear and take their concerns seriously. Dealing with emotional overwhelm upfront is one reason IFS doesn’t require stabilization before attending to trauma. If overwhelm is not a threat, protectors relax and the client’s Self remains present and available.

VALIDATING AND ADDRESSING A PROTECTOR'S FEAR OF EXILE OVERWHELM

THERAPIST: Let's thank this part for sharing its concern. Let it know that we can ask the one who feels terrible not to overwhelm.

- *Exiles generally agree not to overwhelm when they realize that separating and being with the Self is the best way of getting attention.*

THERAPIST: Did you hear? The part says it will not overwhelm. Can we talk to it now?

- *Once the exile agrees not to overwhelm, we check back with protectors to get their permission to proceed.*

Here is an example:

ASKING AN EXILE NOT TO OVERWHELM

Find

THERAPIST: It seems like you have an anxious part who keeps jumping in, do you notice that?

PETE: Yes, it's having a hard time staying back. It won't stay in the waiting room where we put it.

- *The part is at least unblended enough so that Pete can notice it.*

Focus

THERAPIST: Ask what it's upset about.

- *Being curious.*

PETE: It's afraid I'm being taken over.

- *This is the typical protector fear of exile overwhelm.*

THERAPIST: Would it say more?

Protector Fears

PETE: It's afraid I'll be taken over by the feelings of the part who was criticized by my football coach. It says we're messing with something I can't handle.

- *This common fear is another way of describing exile overwhelm.*

Befriend

THERAPIST: How old does it think you are? And don't filter the answer just say what comes up.

- *His answer will likely reveal the exile, the part who is being protected.*

PETE: It thinks I'm 17 years old. That's when I played football.

THERAPIST: Let's give it a chance to get to know the Pete of today, the Pete who's not a part. Ask it to look you in the eye and let you know who it sees.

- *Eye contact is a great strategy for detecting parts – or the Self. (The authors thank Mike Elkin for this insight.)*

PETE: Wow, it's really shocked. It had no idea that I'm grown up.

THERAPIST: How does it respond to you?

PETE: It sees me but still thinks the feelings are too intense for me.

- *This protector has noticed Pete as a grown-up but is not yet in connection with Pete's Self.*

Direct Access

THERAPIST: Can I talk directly to the part who felt criticized by the coach? I'll ask it not to overwhelm.

- *The therapist offers to address the protector's concern (exile overwhelm) by speaking directly to the exile. Alternatively, the therapist could have stayed with introducing the part to Pete's Self and having Pete ask the exile not to overwhelm. Either way is fine but direct access is often faster.*

PETE: That would be great.

THERAPIST: So I want to talk directly to the part who was criticized by that football coach. Are you there?

PETE: Yes.

- *This is the exile speaking.*

THERAPIST: I hear you have some strong feelings, is that correct?

PETE: Yes.

THERAPIST: We can help. If you don't take Pete over, you'll be able to show him what happened and we'll help you to unload this pain for good.

PETE: How?

THERAPIST: If you stop overwhelming Pete, the worried part will be able to relax and then Pete can help you – the Pete who’s not a part. Want to try that?

PETE: I’d like to. But I’m not sure I can.

- *Exiles are capable of separating but are not in the habit of doing so voluntarily because they associate separation with being exiled.*

THERAPIST: That’s fine. Let’s practice.

- *Practice normalizes this new option of being separate yet in relationship.*

PETE: Okay.

THERAPIST: Let’s start with you sharing just a little bit of your feelings at a time. Say 10 percent or, if it prefers, a couple of drops.

- *These are two ways in which an exiled part can share its feelings slowly. Another way is to use the image of a volume dial that can be turned up or down. (The authors thank Michi Rose for the “few drops at a time” technique.)*

THERAPIST (continuing): Are you ready?

PETE: Yes.

THERAPIST: Great. Let me check with Pete first. Pete, did you hear that?

- *Bringing the client’s Self in on the conversation.*

PETE: Yes.

THERAPIST: Did the worried part hear, too?

PETE: Yes.

THERAPIST: Are you both ready for this part to share 10 percent of his feelings?

- *Asking for permission.*

PETE: Yes.

THERAPIST: Okay. Pete and the worried part are both ready for you to share 10 percent of your feelings with him. Go ahead... Let me know when you're feeling it, Pete.

- *Practicing.*

PETE: I'm feeling it.

THERAPIST: Is this level of feeling okay?

PETE: Yes.

Befriend

THERAPIST: Let the part know it's okay so far. What is it like for the part to share this with you?

PETE: A little bit of a relief.

THERAPIST: Great. Is it okay to keep going? Let him know what percentage of the feeling he can share with you now. We can go very slowly. Let me know when you're ready to stop and talk about what it's like for you to be together.

- *Facilitating the relationship with Pete's Self.*



As this example illustrates, we get permission to talk to the exile so we can ask it to stop overwhelming in return for getting some help. The therapist's Self also helps the client's system feel safe enough to experiment.

SCIENCE: ADDRESSING FEARS AND WORKING WITH EXTREMES

The last step with protectors involves understanding their fear, which is at the core of their resistance to change. We ask, "What are you afraid would happen if you didn't do this job anymore?" because the answer either reveals the wound ("I would be alone") or a polarization

(“the suicide part will take over”). When we are too critical or pushy and demanding with protectors their fear will naturally intensify and their behavior will become more entrenched. In this regard, neuroscience can remind us how to pitch therapeutic interventions for best effect.

We find that assessing whether the symptom (the protective part) is rooted in *sympathetic activation* or *parasympathetic withdrawal* is important. For example, rage, panic, flashbacks and binge drinking are generally associated with activation and are rooted in the sympathetic nervous system, whereas numbness, dissociation, and feeling shamed or being highly intellectual are associated with blunting and withdrawal, which are rooted in the parasympathetic nervous system. Some reactions can signify either hyper- or hypoarousal and need to be sorted out before settling on an intervention. For example, a suicidal part can be intense and impulsive (hyper) or quietly looking for an exit to avoid the pain (hypo).

We therapists should always be aware of the reactivity of our own parts and keep them in check. For example, when you are activated do you get nervous or intellectual or controlling? When a client withdraws, do you try too hard? Do you disconnect? Or do you get angry? Whatever you notice about your reactions, if you are able to help your parts relax and trust your Self to be present with the client, good things will naturally unfold.

HANDLING ACTIVATION

As we know, hyperarousal signals danger by mobilizing the body, rousing intense feelings and turning off the areas of the brain that are responsible for assessing, attending, and responding appropriately to our experience. In the face of hyperarousal we should stay calm, rational and non-reactive so that we can help our clients put feelings into words and gain perspective about their present state. When clients are unable to separate from an activated part, we talk directly to the part (direct access) and become the auxiliary brain (the Self of the system), listening to it and addressing its concerns.

This is a *top-down* (accessing the cognitive first then moving toward feelings and, last, physical sensations) approach to a client’s sympathetic hyperarousal (Anderson, 2016). We are there to offer a reasonable response, conveying confidence and clarity when she is overwhelmed. We demonstrate strength but we do not try to control her activated part.

If a client has at least some awareness of being activated, he can use out-of-office interventions that feature any number of simple top-down strategies to help him shift states. He can, for example, check his email, watch TV, read a book, or talk to a friend.

We say: “Mainly, when your protective parts are activated don’t make decisions or do anything regarding the problem at hand, wait until your brain settles down and your parts can give you the room to have perspective again.”

STAYING CALM AND HELPING AN ANGRY PART UNBLEND

John came to a session angry with his partner and eager to talk about an argument from the night before.

JOHN: I'm so pissed at her! She constantly puts our son Jake in the middle of everything, forcing him to choose between us. Why doesn't she get how destructive this is?

THERAPIST: Sounds hard. And based on your history, I'm guessing this dynamic is particularly challenging for you.

- *Validating.*

JOHN: It makes me want to kill her or kill myself. I can't decide which to do first.

- *A scared part is activated in the therapist, who silently asks the part to relax.*

Find and Focus

THERAPIST: I hear you feel really mad. I wonder if the angry part of you would separate a bit?

- *Asking a hyperaroused part to unblend.*

JOHN: Are you serious? I guess you can't handle my feelings any better than her.

- *The angry part is unwilling to separate, which is not unusual for angry parts.*

Direct Access

THERAPIST: I think I'm talking directly to the angry part, right? That's fine with me.

I hear that you're mad because John's son is being put in the middle just like John was with his mom and dad, is that right?

- *Moving right into direct access with the angry part – and John sits back in his chair.*

THERAPIST (continuing): If you can bring the intensity down a bit, I promise he'll come up with a workable solution.

- *Negotiating with the angry part, helping it to separate from John a bit. After a few moments John's shoulders relax and his head tilts.*

JOHN: I know you're right. Thanks for sticking with me.

- *The angry part has made room for John's Self and his sympathetic activation comes down.*



This vignette illustrates the therapist's need to keep his own parts in-check while helping reactive parts in the client unblend. He moves into direct access to remain Self-led and available to an angry, threatening part who is not immediately willing to unblend. Staying confident and meeting the part head-on without trying to control it, helps it to relax.

BEING CALM AND NONREACTIVE WHEN A CLIENT IS HYPERAROUSSED

Noah sits down and immediately begins to describe an interaction with his adolescent son over the weekend.

NOAH: I was so mad at him. I really lost it. I had been very clear about the rules. When we are not home no friends in the house without our permission and absolutely no drinking. I got a call from the police because our neighbor reported noise coming from our house. I was beyond mad.

THERAPIST: I can see how upset you are as you tell the story now.

NOAH: Wouldn't you be angry? I am still so embarrassed.

THERAPIST: Of course that would be upsetting. Let your angry part know that its response makes sense.

- *Validating the part.*

NOAH: I'm glad you get it.

Find and Focus

THERAPIST: If the angry part is willing to separate just a little bit we'll be able to hear it even better.

- *Unblending*

NOAH: You aren't afraid of my feelings?

THERAPIST: Not at all. I know your feelings have important information for us.

- *Continuing to validate the part. After a moment, Noah's voice and body posture soften.*

NOAH: No one could tolerate my feelings when I was a kid. I would never hurt anybody. I just feel things intensely.

Befriend

THERAPIST: Let the angry part know that its feelings are fine with us and we want to hear all about it.

- *Welcoming a part who alarms others and has experienced disapproval.*



Angry parts are particularly sensitive to rejection - and used to it. But, as this example illustrates, they calm down and are often grateful when they are welcomed with genuine concern and interest.

ACCESSING THE COGNITIVE MIND WHEN A CLIENT IS HYPERAROUSSED

The therapist goes to the waiting room to get Tina for her 1:00 pm appointment. She is clearly anxious. The therapist greets her and as they start heading back to the office Tina starts talking, unable to wait.

TINA: My husband is having an affair!

THERAPIST: When we get into the office you can tell me more.

TINA: Oh my god, I'm devastated!

THERAPIST: We're almost there. I want to hear what's going on.

- *The therapist closes the office door.*

THERAPIST (continuing): Tell me what's happening.

TINA: I saw a text message on his phone last night from a woman.

THERAPIST: What did it say?

TINA: I can't wait to see you tomorrow.

THERAPIST: That's it?

TINA: Yes. Who is this woman? Why is he cheating on me?

Implicit Direct Access

THERAPIST: Let's slow down and figure this out together. Did you ask him about the text?

- *Rather than asking to speak to Tina's panicked part, the therapist simply begins to speak to the part directly, acting as the Self of the therapeutic system, in contrast to "explicit direct access" in which we ask for permission to talk to the part directly, we call this "implicit direct access."*

TINA: No of course not. Then he'll know I know.

THERAPIST: Did you see any other text messages from this person?

TINA: No.

THERAPIST: What makes you sure he's having an affair?

TINA: What else can it be?

Find

THERAPIST: Well, it could be a colleague from work. It could be an old friend or someone your husband plans to meet in a platonic way. Who knows? But I see you have a really activated part and I wonder if it's willing to give you some room?

TINA: How can I relax when my whole marriage is on the line?

Going Inside

THERAPIST: I'm hearing that this part doesn't want to relax right now. Can I ask a question? Are you hearing from parts who have a different perspective?

- *Tina listens internally and her body relaxes.*

TINA: Well, actually, as I was driving here today it did cross my mind that this could be a misunderstanding.

THERAPIST: What does that part want you to know?



As this example illustrates, even as the perceptions of some parts are seriously skewed by past experiences we can ask for other inner perspectives. Parts who are calmer—along with the client's Self—are there and will be able to contribute their observations once panicked parts get help to unblend a bit.

Top-down strategies for unblending sympathetic activation

1. Provide a rational perspective. Help the client to *make sense* of her reaction.

2. Validate the client's experience and put feelings into words: "I'm guessing you feel..."
3. Be clear, centered and express care (compassion).
4. If these strategies don't help the part unblend, move on to direct access. (Anderson, 2016)

HANDLING WITHDRAWAL

At the opposite end of the spectrum, when increasing danger activates the dorsal branch of the parasympathetic nervous system, shutting down several key structures in the brain, hypoarousal kicks in and the client becomes disconnected from his body, feelings and rational mind (Anderson, 2016). This signals an increased risk of serious harm.

Hypoarousal, a state of over-inhibition (Lanuis, 2010), calls for a very different set of therapeutic interventions. Originating from lower, more primitive brain structures, hypoarousal can occur on a continuum, with one client having little access to body sensations but still being able to identify emotions and thoughts, to another client being fully disconnected from body and feelings but having access to the intellect (Anderson, 2016). Highly cognitive clients often operate primarily from their intellectual parts but are cut off from sensations and feelings. With hypoarousal we use bottom-up interventions, focusing first on body sensations, next on feelings and finally on beliefs to help these parts unblend.

Here are questions to help you assess a client's level of blunting.

- "Are you aware of your feet touching the floor?"
- "Can you take a deep breath?"
- "Can you look at me?"
- "Can you put into words what you are feeling right now?"

In response to withdrawal we want to give alarmed parts as much time, space and control as they require, because it takes longer to recover from withdrawal than it does to recover from activation (Porges, 2011). If the client appears totally shut down, we again become the auxiliary brain by moving right into direct access, which helps the client shift to a mindful state more quickly.

SLOWING DOWN AND USING DIRECT ACCESS WITH HYPOAROUSAL

When Ariel was in the middle of a session the door to the office opened and a loud voice asked, “Where is Dr. Johnson’s office? This is my first time here and I am lost.”

Ariel froze, the door closed.

The Therapist is the Self of the Therapeutic System

THERAPIST: Are you okay? Can you hear me, Ariel? Can you look at me?

- *No response, which tells the therapist that her dorsal parasympathetic nervous system is activated.*

THERAPIST (continuing): Can you let me know what’s going on inside right now? I see you’re struggling. Can you take a deep breath?

- *Ariel’s continuing lack of response tells the therapist that she has shut down and is not feeling safe.*

THERAPIST (continuing): Can you move your finger?

- *She does.*

THERAPIST (continuing): Great.

- *Since she can move her body she is not totally offline, the therapist starts talking directly to the part who has taken over.*

Direct Access

THERAPIST (continuing): I’m going to speak to the part who took over. I’m here. I’m with you. I’m not going to push you in any way. We can take as long as you need to feel safe. You’re the boss and I trust you.

- *After a few minutes Ariel takes a breath and the therapist knows she’s back. She adjusts and looks around.*

ARIEL: I went into the closet. It was dark and quiet and I could hear you in the distance but I thought if I stayed quiet no one would find me.



As we see here, some clients with trauma histories go into a state of parasympathetic blunting in response to threat. We know that a part has taken evasive action inside, and if we speak to it directly, reassuring it without engaging in any kind of pressure, the client will be able to return.

Parasympathetic hypoarousal requires bottom-up strategies that help clients unblend and shift states. Being too active, too involved or too direct may scare the part and cause it to dig in with further disconnection. Out-of-office strategies for hypoarousal are emotion- or body-based, such as going for a walk or run, trying some yoga poses, gardening, having sex, listening to music or watching an emotionally charged movie.

Bottom-up strategies for unblending parasympathetic withdrawal

1. Assess your client's level of withdrawal: "Can you hear me? Can you take a deep breath?"
2. Help clients to *sense* (not make sense of) their reaction by focusing first on the body, then on emotions and last on beliefs.
3. Express connection and nurturance (empathy).
4. Let go of having an agenda: slow down, express trust.
5. If these strategies are unsuccessful, move on to direct access. (Anderson, 2016)

Exercise

THERAPIST'S PARTS WITH EXTREME PROTECTORS

DIRECTIONS: Try this exercise with a client whose “activated parts” trigger you and with a client whose “blunted parts” trigger you.

Think of a client who triggers you.

- Bring this person up in your mind’s eye.
- Put the person into a room that has a window and a lock on the door.
- Stay outside the room, looking at your client through the window.
- Now see the client do the thing that triggers you.

Notice your parts who get activated and write them down here:

See if your triggered parts are willing to separate enough so that you can hear them clearly.

Notice if your client’s parts seem activated (hyperaroused) or withdrawn (blunted).

Notice if your parts get activated or withdrawn in response to your client’s parts.

Now ask your parts to stand behind you for a moment and watch while your Self stays with your client as her or his parts do the triggering thing.

What was it like for your parts to witness your Self being with your client in that moment?

Would your parts be willing to let you be with your client the next time you see them?

When this feels complete, invite your client out of the room and thank your parts for showing up to share their experience with you.

DIRECT ACCESS

Clients who have experienced extreme attachment rupture often have protectors (and sometimes exiles) who don't trust anyone. These parts frequently hide the Self deeply within or outside the body for safekeeping. To feel comfortable about therapy, they need recognition, control and a direct relationship with the therapist. We know this is the case when protectors won't unblend or refuse to talk, or when exiles repeatedly overwhelm the client.

How Direct Access Differs from Internal Communication

Unlike internal communication, which is a three-way conversation among therapist, client's Self and client's parts, direct access is a two-way conversation. The Self of the therapist speaks directly with the client's target part, asking periodically if the part will permit the client's Self to come into the conversation.

In the two-way conversation of direct access, we don't need to take the first four steps (find, focus, flesh out, feel toward), we simply start with the Self of the therapist talking to a part of

the client. When we use direct access we begin at the 5th step: we befriend the part before going on to explore its fears in the 6th step.

How to Do Direct Access

To begin, the therapist asks for permission to speak to the part. For this example we use a dissociative part.

- “Can I talk directly to the part who takes you out?”

The client nods.

- “Great. I want to talk to the part who takes Margaret out. Are you there?”

5TH STEP: BEFRIEND

Then we want to learn about and develop a friendly relationship with the target part.

- “What do you do for Margaret?”
- “How did you get the job of taking her out?”
- “Whom do you protect?”
- “What do you want Margaret to know about you?”
- “How do you feel about doing this job?”
- “For how long have you done this job?”
- “How old are you?”

6TH STEP: FEARS

And, finally, we inquire about protector fears, the prime obstacle to progress in therapy.

- “What are you concerned would happen if you stopped doing this job?”

As we mentioned in relation to eliciting fears when we use internal communication, protectors have a number of typical concerns:

1. The part will disappear if it isn’t needed in this job.

2. The therapy will end and the relationship with the therapist will be lost if protectors allow the client's Self to show up.
3. A secret will be revealed.
4. The client will be overwhelmed with pain.
5. The therapist won't be able to handle the exiled pain.
6. A polarized protector will take over if this part relaxes.
7. Self-energy is dangerous and will attract punishment.
8. There is no Self.
9. The therapist – or other parts – will judge this protector for the damage it has done.
10. Change will destabilize the client's inner system.

When we do direct access, we listen until we understand the part's role in the system and then we validate the importance of its work, which was crucial for survival at one point in time no matter how deleterious its effects may have become. Following are three examples of direct access:

- The first illustrates direct access plain and simple.
- The second illustrates how we segue from internal communication to direct access.
- The third illustrates how we use direct access with polarized protectors who refuse to unblend unilaterally.

GETTING TO KNOW A PROTECTOR WITH DIRECT ACCESS

THERAPIST: Can I speak directly with the part who avoids parties?

CLIENT: Yes.

THERAPIST: Okay.

- *Having gotten permission, the therapist now addresses the part directly.*

THERAPIST (continuing): I would like to speak with the part who avoids parties. Are you there?

AVOIDANT PART: Yes.

Protector Fears

THERAPIST: What are you concerned would happen if you let Joe go to parties?

PART: First he would embarrass himself and then he would criticize himself for days.

- *The avoidant part is naming other parts.*

THERAPIST: You mean he has a part who would feel embarrassed and a part who would criticize him?

PART: No. I mean he has a part who would be embarrassing and a part who would criticize him afterward.

THERAPIST: I see. What would the embarrassing part do?

PART: Blab. Say too much. Say the wrong thing. Freak people out.

THERAPIST: Would you let Joe find out more about the embarrassing part if it promised not to take him over?

- *The therapist wants to find out more about this embarrassing part. Is it an exile who, looking for help, reveals intimate details about Joe's life to strangers? Is it a protector with an agenda, like trying to recruit people to take care of Joe's exile?*

PART: I suppose so. Though I don't know why he'd bother.

- *The avoidant part clearly has a negative view of the "embarrassing" part.*

THERAPIST: He could help the embarrassing part by finding out if it's protecting someone else or if it needs help.

PART: Hmmm. I hadn't thought of that.

THERAPIST: With your permission I can show Joe how to help the embarrassing part either way. And if it doesn't work you can still avoid parties.

PART: Okay you can try.



In the beginning, trying Direct Access with clients can feel strange and a bit awkward, but it's a great way to learn about protector fears, particularly when we are dealing with protectors who are wary and unlikely to cede any control. As we learn here, Joe's avoidant part also had a keen eye on other extreme parts, so talking to it directly revealed a crucial inner dynamic we were unable to access with internal communication.

MOVING FROM INTERNAL COMMUNICATION TO DIRECT ACCESS

Find: Internal Communication

THERAPIST: You have a part who likes to be in control and sees itself as being you. I sense that part is in the driver's seat right now. Are you aware of it?

- *Checking on the level to which this part is blended with the client.*

GABRIELA: Not really. It just feels like me.

- *The part is highly blended.*

Turning to Direct Access

THERAPIST: I hear that. I'm guessing this part is pretty active in your life. I'm wondering if you would be willing to let me to talk to it directly so we can better understand its intentions.

GABRIELA: Sure I guess so. I don't really notice that it's a part.

Direct Access

THERAPIST: Thanks. So I want to talk to the part of Gabriela who runs her daily life and keeps things under control. Are you there?

GABRIELA'S MANAGER PART: Somebody has to be in charge.

THERAPIST: Tell me more about someone needing to be in charge.

GABRIELA'S MANAGER PART: Well, someone needs to step up to the plate and run things.

Befriend

THERAPIST: So you're the part who steps up to run things, is that it?

GABRIELA'S MANAGER PART: Yes someone has to.

THERAPIST: What do you mean someone has to?

GABRIELA'S MANAGER PART: She gets overwhelmed all the time. She has a hard time handling most anything. She just checks out.

THERAPIST: I get it! Sounds like you have an important job. Can you tell me where you are located in, on or around Gabriela's body?

- *Validating the part and then locating it in the client's body.*

GABRIELA'S MANAGER PART: I'm everywhere. I have to be. But mostly I'm in her head I guess. As I said before someone has to be in charge.

Protector Fears

THERAPIST: What would happen if you didn't run things and keep them under control?

GABRIELA'S MANAGER PART: I couldn't even imagine not running things. I've been in charge for so long.

Befriend

THERAPIST: I was wondering about that. Do you know when you first took on this job?

GABRIELA'S MANAGER PART: Don't you dare mess up!

THERAPIST: Who said that?

GABRIELA'S MANAGER PART: Her dad yells at her all the time. She did it wrong or she's not as good or as smart as her older sister. He was in the military before he met her mom and he was strict.

THERAPIST: This is really making sense. Let me see if I'm getting it right. You are the part of Gabriela who runs things, keeps things in control, always trying to do the right thing so that the little girl who got yelled at and criticized by her dad won't make any more mistakes. Is that right?

- *Exploring its job.*

GABRIELA'S MANAGER PART: Exactly. Someone had to help her.

THERAPIST: I totally get that. You had a really important role in keeping that little girl from getting in trouble.

- *Validating the part's job.*

GABRIELA'S MANAGER PART: You got it.

THERAPIST: What if I were to tell you that there is another way to solve this problem and that you would not have to keep working so hard? Would you be interested in hearing more?

- *Inviting the part to try something new.*

GABRIELA'S MANAGER PART: I can't even imagine not doing this job for her.

THERAPIST: Well that would be completely up to you. But if you give us access to the little girl, Gabriela can help heal those wounds and keep her safe so you won't have to keep doing this job.

GABRIELA'S MANAGER PART: That would be great but I doubt it's possible.

THERAPIST: I understand. I know how to do it, but I need your permission to access the little girl.

- *Expressing confidence and getting permission.*

GABRIELA'S MANAGER PART: I am open to that.

THERAPIST: Great. I appreciate you sharing all this with me. It is super helpful.

GABRIELA'S MANAGER PART: I'm glad someone was willing to listen.

Returning to Internal Communication

THERAPIST: Okay to bring Gabriela back? Gabriela, did you hear all that?

GABRIELA: Wow, I did. I had no idea that part was trying to protect that little girl. I appreciate that!

THERAPIST: Let her know.

GABRIELA: She likes the recognition.



When we use direct access because a protector is stubborn about staying blended, we have a special opportunity to make friends with it. Even silent parts often speak freely when invited. As we see with Gabriela, the most stubborn protectors are also often the most vigilant and heroic.

USING DIRECT ACCESS WITH POLARIZED PROTECTORS WHO WON'T UNBLEND

Camille's off-and-on boyfriend called from another state one spring day to propose to her. She was walking outside on a lunch break. She described stopping and screaming at him over the phone as pedestrians and colleagues walked by looking shocked and puzzled.

CAMILLE: Who does he think he is?

- *Camille is blended with the angry part right now.*

Find

THERAPIST: Who needs your attention, Camille?

CAMILLE: He is such an asshole. I moved across the country to be with him and now when I'm back here he calls to propose!

- *The angry part ignores the invitation.*

THERAPIST: This part?

- *The therapist continues to invite the client's Self.*

CAMILLE: I am so embarrassed. Do you know how many people were in front of my building walking around on Monday. It was a beautiful day. It was lunch hour!

- *This is a polarized protector, a critic.*

THERAPIST: That part?

- *The therapist offers another invitation to the client's Self.*

CAMILLE: I'm not going to get that promotion. I might as well have been lying on the ground asking people to step over me.

- *The critic also ignores the therapist's invitation.*

THERAPIST: I hear two parts who I'm pretty sure we've heard from before. There's the part who is angry with Matthew and there's the part who scolds you for being angry. Which of them needs your attention first?

- *The therapist names the polarity and again invites the client's Self – which of course requires parts to be willing to unblend.*

CAMILLE: He's a no-good son-of-a-bitch. I'm taking his contact information off my phone.

- *The angry part jumps back in. None of these parts is willing to unblend.*

Direct Access

THERAPIST: How about if I speak with these parts directly? I want to talk to the part who is mad at Matthew and the part who criticizes Camille for being mad. Are you both there?

CAMILLE: Yes

THERAPIST: Would you be willing to take turns?

- *The therapist asks them to cooperate.*

CAMILLE: Sure.

Direct Access with Two Parts

First Part:

THERAPIST: I want to talk to the part who criticizes Camille first. What are you concerned would happen if you stopped doing that?

- *The therapist chooses to talk to the critic first since the angry part probably will not cooperate until the critic does.*

CAMILLE'S CRITIC: She'd alienate everyone.

THERAPIST: That would be bad. We don't want her to alienate everyone. If she could help the part who feels hurt and she didn't have to get so mad would you need to criticize her this way?

- *The therapist validates the critic's concerns and offers a new option.*

CAMILLE'S CRITIC: She'll always get mad.

- *The critic does not acknowledge the client's Self and seems only to see the angry part.*

THERAPIST: I'm going to talk to the angry part in a minute. But that's not who I mean. I'm talking about the Camille who's not a part. She can help the angry part and the part who feels hurt. Do you know her?

- *The therapist introduces the idea that Camille has a Self.*

CAMILLE'S CRITIC: No.

- *The therapist asserts that the Self can help both protectors equally.*

THERAPIST: How about I talk to the angry part now and then both you and the angry part can meet the Camille who's not a part?

CAMILLE'S CRITIC: Okay.

Second Part:

THERAPIST: Thanks. Now I want to talk to the angry part. Are you there?

CAMILLE'S ANGRY PART: Yes.

THERAPIST: Who do you protect?

CAMILLE'S ANGRY PART: Camille.

THERAPIST: How old is she?

- *The answer "Camille" means the angry part protects an exile.*

CAMILLE'S ANGRY PART: Sixteen.

THERAPIST: So there's a 16-year-old who needs help. With your permission, Camille's core, the Camille who is not a part can help her. Then you won't have to work so hard. Would that be good?

- *The therapist validates the angry part's concerns and offers a new option.*

CAMILLE'S ANGRY PART: Yes.



Sometimes polarized protectors simply take turns blending as they argue. The result can look to the casual observer strangely like an argument between two people in conflict who are somehow housed in the same body - which, from the IFS perspective, is the case.

Once Camille's polarized parts agree, they can meet her Self and – moving into internal communication – Camille can help heal the exile so both parts can let go of their protective roles.

Exercise

EMBODYING A PART

DIRECTIONS: First find a protective part to check in with and try direct access on yourself by embodying this protector.

Invite the part to embody and show you what it does for you.

- The part may want to move in a certain way.
- It may want to speak (or sing, shout...).
- It may have a particular posture or facial expression it wants you to notice.
 - Ask the part if you're getting it right.

Ask how long it has helped you in this way.

Ask what would happen if it didn't do this job for you.

Ask the part if you are fully getting its role or is there more.

Ask if you can help the part it protects.

Exercise

DIRECT ACCESS

DIRECTIONS: Direct access is a two-way conversation in which the Self of the therapist speaks directly with the client's target part, periodically asking if the client's Self can be brought in on the conversation. In direct access, the therapist speaks to the part.

You can use direct access with clients or, for practice, you can conduct a role-play with a colleague or embody one of your client's protective parts by going back and forth between being the therapist and the client, either changing chairs or shifting a bit to one side and then the other on a couch. Try this exercise with a protector.

1. "I want to talk to this protective part directly. Are you there?"

2. "What do you do for _____ [Marianne]?"

3. "How long have you had this job?"

4. "How's it going?"

5. “What are you concerned would happen if you stopped?”

6. “If we could help that part (the one who would take over and do something problematic if this part were to stop), would you still need to do this job?”

7. “Have you met the _____ [Marianne] who’s not a part?”

8. “Would you like to meet her? She can help you and the parts you’re worried about.”

9. “Will you give _____ [Marianne] permission to help the part you’re worried about if it agrees not to take her over?”

OUR INVITATION TO PROTECTORS

Whether we use internal communication or direct access, once we have gone through the 6F's with a protector, we expect it will feel that the client's Self has a good sense of its job and its fears. If we have succeeded thus far, we have a relationship with the part and we can offer it an alternative solution to the underlying problem of the exile. Our invitation to hard-working protectors, whose efforts have usually brought mixed results at best, is to *try something new*. As incentive, we assert that the part's *risk in trying something new will be minimal* ("you can always go back to doing what you do"), the *wound can be healed* and *the part can be safe* with the client's Self.

WHAT WE SAY TO PROTECTIVE PARTS

"If this little girl no longer felt the pain of being alone in the world would you need to protect her?"

"There is a way to heal her. And once she is healed you would be free to do other things. Are you interested?"

Protective parts usually welcome the opportunity to give up the roles they were forced into once upon a time. We assure them that rather than wanting to get rid of them we want to help them get rid of their jobs. In this way, we offer them a new possibility and hope for a different future.

THE 6F'S GOAL

First we *find*, *focus* on and *flesh out* a protective part in order to help it unblend and notice the client's Self. Next we ask how the client feels toward the target part in order to help other parts unblend so the client's Self is available. Finally we *befriend* the target part, explore its *fears* and *invite* it to try something new. Our goal with protective parts is to get their permission to access and heal exiled parts.